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Question 1

Not answered

Marked out of 4.00

Epidemiology of personality disorders

For each of the settings below, choose the most frequently encountered personality disorder from the given list.

Community / general population	Choose...
Primary care / GP services	Choose...
Secondary psychiatric care	Choose...
Prison services	Choose...

Your answer is incorrect.

Explanation:

Cluster C personality disorders are the commonest personality disorders to be encountered among primary care attenders (Moran et al. 2000). When ICD diagnoses are considered individually, anankastic personality is the most common type in primary care, followed by impulsive (emotionally unstable) type.

In general, the prevalence of personality disorders among psychiatric outpatients and inpatients is high, with many studies reporting a prevalence of greater than 50% of samples. Borderline PD is generally the most prevalent (and certainly most heavily researched) category in psychiatric settings.

Dissocial personality is the most prevalent category of personality disorder in prison settings.

The correct answer is: Community / general population → Anankastic personality, Primary care / GP services → Anankastic personality, Secondary psychiatric care → Emotionally unstable - borderline personality, Prison services → Dissocial personality

Question 2

Not answered

Marked out of 3.00

PTSD Management

A 34-year-old policeman was involved in rescuing victims of a terrible accident recently. He started experiencing flashbacks of it later, and currently experiences low mood, poor sleep and has avoided discussing the accident with his wife. She is concerned as he is increasingly withdrawn and is put off from having sex with her. Choose one best intervention from the given options.

He comes to you 3 days after the accident.

Choose...



He comes back after 3 months with the same symptoms persisting.

Choose...



Which one among the above is shown by an RCT to be leading to increased risk of developing the disorder?

Choose...



Your answer is incorrect.

Explanation:

The current NICE guidelines on PTSD could help answer this question.

In those with symptoms of PTSD within 4 weeks of trauma - watchful waiting is advocated; for those who present within 3 months, trauma focused CBT is advocated; patients that have symptoms for more than 3 months are said to have chronic PTSD - either trauma focused CBT or EMDR (the only choice here) can be used. If these measures fail, then offer pharmacological treatments (paroxetine or mirtazapine from primary care [GPs], amitriptyline or phenelzine under specialist care).

NOTE: Single debriefing is contraindicated as per a Cochrane review.

The correct answer is: He comes to you 3 days after the accident. → Watchful waiting, He comes back after 3 months with the same symptoms persisting. → EMDR, Which one among the above is shown by an RCT to be leading to increased risk of developing the disorder? → Single debriefing

Question 3

Not answered

Marked out of 5.00

Aetiology of common disorders

Which of the listed risk factors is most strongly associated with the following clinical situations?

A 24-year-old woman with depressed mood, guilt feelings, feelings of worthlessness (as a mother), auditory hallucinations and mood congruent delusions

Choose...

A 73-year-old gentleman with visual hallucinations. On examination his cognitive status remains intact and he does not exhibit any psychotic phenomenon

Choose...

A 23 year old gentleman suffered relapse of his psychotic symptoms despite adherence with medications and tested positive for drugs

Choose...

A 20-year-old young woman presenting to the A&E with chronic feelings of emptiness, fear of abandonment and recurrent episodes of self-cutting.

Choose...

A 77-year-old gentleman with history of tremors, rigidity and bradykinesia for the last three years.

Choose...

Your answer is incorrect.

Explanation:

Case 1 refers to postnatal depression, which occurs in women during first 6 months following childbirth.

Case 2 describes an elderly man with Charles-Bonnet syndrome. This syndrome occurs due to reduced visual acuity in old age and is seen in patients with senile cataracts, senile macular degeneration etc.

Case 3 is suggestive of relapse of psychosis, which is quite common due to a high rate of abuse of illicit substances like cannabis.

Case 4 refers to borderline personality disorder for which one of the important epidemiological associations is childhood sexual abuse.

Case 5 is a gentleman with Parkinson's disease. Recent studies suggest that too much manganese, an essential element required by the body in tiny amounts but toxic at elevated levels, may contribute to the early development of Parkinson's disease symptoms in susceptible people.

The correct answer is: A 24-year-old woman with depressed mood, guilt feelings, feelings of worthlessness (as a mother), auditory hallucinations and mood congruent delusions → Having given birth 6 months previously, A 73-year-old gentleman with visual hallucinations. On examination his cognitive status remains intact and he does not exhibit any psychotic phenomenon → Reduced visual acuity, A 23 year old gentleman suffered relapse of his psychotic symptoms despite adherence with medications and tested positive for drugs → Cannabis abuse, A 20-year-old young woman presenting to the A&E with chronic feelings of emptiness, fear of abandonment and recurrent episodes of self-cutting. → Childhood sexual abuse, A 77-year-old gentleman with history of tremors, rigidity and bradykinesia for the last three years. → Manganese exposure



Question 4

Not answered

Marked out of 3.00

Risk factors and etiology

For each of the settings below choose the most strongly associated risk factor from the given list.

A young man with social withdrawal and mood incongruent delusions and hallucinations.

Choose...



A young woman living in South London with sleep disruption and weight loss and low self esteem

Choose...



A 33-year-old lady with affective instability, self-harm and intense emotions and multiple relationship breakdowns.

Choose...



Your answer is incorrect.

Explanation:

Case 1 suggests a diagnosis of schizophrenia. Known risk factors include immigration.

Case 2 is a classical description of a subject studied by Brown & Harris in relation to certain "vulnerability factors"--namely early maternal loss, lack of a confiding relationship, greater than three children under the age of 14 at home and unemployment-that can interact with "provoking agents" to increase the risk of depression.

Childhood sexual abuse has an established epidemiological association with emotionally unstable personality disorder (Case 3).

The correct answer is: A young man with social withdrawal and mood incongruent delusions and hallucinations. → Immigration, A young woman living in South London with sleep disruption and weight loss and low self esteem → Early maternal loss, A 33-year-old lady with affective instability, self-harm and intense emotions and multiple relationship breakdowns. → Childhood sexual abuse



Question 5

Not answered

Marked out of 4.00

Personality disorders (1)

For each of the following situations, identify the most appropriate personality disorder from the list:

Tom is a 42-year-old gentleman who complains of depressive symptoms. He states that the health care system has let him down and the GP was not interested in his wellbeing. He is unemployed and has been fired from several jobs in the past. He is single, living with his parents because he cannot trust women. He also mentions that some of his previous arguments have 'got physical'.

Choose...

Tracy is a 23-year-old university student who has history of recurrent depression. During your initial consultation she tells you that she has a new boy friend who is wonderful but none of her flatmates like her. She feels empty inside and has stopped enjoying things. At follow up, she tells that she has a new boyfriend, her last boyfriend was horrible but the new one is very nice and caring. She is feeling well at the moment. She tells you that you have been very helpful and thinks you are the only person she can relate to. When you mention of discharging her, she tells you that she was abused by her father and has often thought of killing him and herself. She thinks she might be gay but is not sure.

Choose...

Robert is a 59-year-old man who insisted on seeing 'an expert' for consultation. He has been treated with SSRI for depression with no positive response at all. He is a well-groomed man who is very articulate. His third wife has left him recently and reports having 6 children with whom he has no contact. They are all doing "exceedingly well". He has recently been made bankrupt and he is facing fraud charges, which he dismisses as 'just the ordinary sort of things that everyone in a business does'. He thinks that a clean suicide would be the way to escape this mess with the most dignity.

Choose...

George is a 28-year-old male who wants to become a gym instructor. He is complaining of shyness and has no confidence. He has never had a girl friend although he is wishing to have a close relationship. He is also afraid of rejection.

Choose...

Your answer is incorrect.

Explanation:

Clinical description of paranoid personality disorder- sensitive, suspicious, pre-occupied with conspirational explanations, self-referential, distrust of others.

Clinical description of emotionally unstable personality disorder, borderline type-unclear identity, intense and unstable relationships, unpredictable affect, threats or acts of self harm, impulsivity.

Clinical description of narcissistic personality disorder- presence of grandiosity, exaggerated sense of own importance, frequently self-centred, intolerant of others, craving for attention and admiration.

Clinical description of avoidant personality disorder- excessive strain and self consciousness in relating to others, fear of negative evaluation by others, timid and insecure, feelings of discomfort in group or social settings, have low self-esteem, may crave affection but experience overwhelming fears of rejection. (

Ref: Oxford handbook of psychiatry- Pg 447)

The correct answer is: Tom is a 42-year-old gentleman who complains of depressive symptoms. He states that the health care system has let him down and the GP was not interested in his wellbeing. He is unemployed and has been fired from several jobs in the past. He is single, living with his parents because he cannot trust women. He also mentions that some of his previous arguments have 'got physical'. → Paranoid personality disorder, Tracy is a 23-year-old university student who has history of recurrent depression. During your initial consultation she tells you that she has a new boy friend who is wonderful but none of her flatmates like her. She feels empty inside and has stopped enjoying things. At follow up, she tells that she has a new boyfriend, her last boyfriend was horrible but the new one is very nice and caring. She is feeling well at the moment. She tells you that you have been very helpful and thinks you are the only person she can relate to. When you mention of discharging her, she tells you that she was abused by her father and has often thought of killing him and herself. She thinks she might be gay but is not sure. → Emotionally unstable personality disorder- borderline type, Robert is a 59-year-old man who insisted on seeing 'an expert' for consultation. He has been treated with SSRI for depression with no positive response at all. He is a well-groomed man who is very articulate. His third wife has left him recently and reports having 6 children with whom he has no contact. They are all doing "exceedingly well". He has recently been made bankrupt and he is facing fraud charges, which he dismisses as 'just the ordinary sort of things that everyone in a business does'. He thinks that a clean suicide would be the way to escape this mess with the most dignity. → Narcissistic personality disorder, George is a 28-year-old male who wants to become a gym instructor. He is complaining of shyness and has

no confidence. He has never had a girl friend although he is wishing to have a close relationship. He is also afraid of rejection. → Avoidant personality disorder



Question 6

Not answered

Marked out of 4.00

Treatments in bipolar disorder

Choose one best management option for each question below:

A 44-year-old woman presents with a relapse after 3 episodes of mania. Presently she has a mixed episode with both manic and depressive features. She is currently medication free as she developed renal side effects with lithium in the past.

Choose...

A 17 year old boy diagnosed with bipolar illness by tertiary services presents with first episode of mania. Choose a drug licensed for this indication.

Choose...

A bipolar patient with treatment resistant depression not responding to quetiapine, olanzapine and antidepressant augmentations. She is already on valproate as mood stabiliser.

Choose...

A 49-year-old man has a history of bipolar disorder. He is not on any medication for the last 2 years. He is now presenting with symptoms of mania. Choose one drug useful for treatment and prophylaxis of bipolar disorder

Choose...

Your answer is incorrect.

Explanation:

Case 1: Sodium valproate should be used in mixed episodes of bipolar disorder and is a good alternative choice for lithium, in patients with renal impairment. Mixed episodes should be treated as a manic episode and the best evidence for efficacy in treating mixed episodes is for sodium valproate, which has been shown to be more effective than lithium or placebo (Mitchell et al 2004).

Case 2: Lithium monotherapy is the first line in this scenario. It is effective in both manic and depressive relapses, although it is more effective in preventing mania. Lithium is indicated and licensed in the treatment of moderate to severe mania with a NNT of 6.

Case 3: Lamotrigine appears to be effective both as a treatment of bipolar depression and as prophylaxis against further episodes. It does not induce switching or rapid cycling.

Case 4: Lithium is useful for both the treatment and prophylaxis of bipolar disorder. It reduces the number and severity of relapses. NICE supports the use of lithium as a first line mood stabilizer.

The correct answer is: A 44-year-old woman presents with a relapse after 3 episodes of mania. Presently she has a mixed episode with both manic and depressive features. She is currently medication free as she developed renal side effects with lithium in the past. → Sodium valproate, A 17 year old boy diagnosed with bipolar illness by tertiary services presents with first episode of mania. Choose a drug licensed for this indication. → Lithium, A bipolar patient with treatment resistant depression not responding to quetiapine, olanzapine and antidepressant augmentations. She is already on valproate as mood stabiliser. → Lamotrigine., A 49-year-old man has a history of bipolar disorder. He is not on any medication for the last 2 years. He is now presenting with symptoms of mania. Choose one drug useful for treatment and prophylaxis of bipolar disorder → Lithium



Question 7

Not answered

Marked out of 4.00

Rehabilitation psychiatry

Choose one option from the below for each of the following descriptions of rehabilitation services.

Service users themselves run a rehabilitation program. Social inclusion is achieved via meaningful daytime activity and education at a common residence.

Choose... ▾

The rehabilitation program owns specified jobs outside the residence. Members of the residence are rotated in these jobs before becoming suitable for long-term jobs.

Choose... ▾

A vocational rehabilitation program works with the aim of placing people in jobs first and then training using job coaches.

Choose... ▾

This is the core of most current psychiatric rehabilitation units which improves rehabilitation results and diminishes the demand for clinical services

Choose... ▾

Your answer is incorrect.

Explanation:

Q1: Clubhouses provide members with a meaningful day, educational opportunities, in-house vocational training and social/recreational options. The clubhouse owns some employment positions which are filled consecutively by clubhouse members, usually over a 6-month period and frequently in a part-time capacity.

Q2: Transitional employment programmes emerged from the clubhouse model developed by Fountain House in 1948. Transitional employment is a time-limited, supported work experience whereby members of the residence are rotated in these jobs before becoming suitable for long-term jobs.

Q3: This is supported employment programme. The individual is placed in a full or part-time job supported by an employment consultant. There is usually a minimum of prevocational training. 'Place and train' principle- not 'train and place' is used; this makes transfer of learning easier (Drake & Becker model).

Q4: Skills training/prevocational training is the core of most current psychiatric rehabilitation units in UK. Skills development improves rehabilitation results, diminishes the demand for clinical services and increases the likelihood of gaining employment.

The correct answer is: Service users themselves run a rehabilitation program. Social inclusion is achieved via meaningful daytime activity and education at a common residence. → Clubhouse, The rehabilitation program owns specified jobs outside the residence. Members of the residence are rotated in these jobs before becoming suitable for long-term jobs. → Transitional employment, A vocational rehabilitation program works with the aim of placing people in jobs first and then training using job coaches. → Supported employment, This is the core of most current psychiatric rehabilitation units which improves rehabilitation results and diminishes the demand for clinical services → Prevocational training



Question 8

Not answered

Marked out of 3.00

Personality disorders (2)

Choose one option from the given list that is most appropriate for each clinical description below:

A 42-year-old woman has always been extremely neat and conscientious; she often stays long after normal working hours to check on punctuation errors. She had to do all jobs by herself and would not delegate her work.

Choose...

A 35-year-old man refuses to provide answers to standard questions during an initial clerking and threatens to stop the interview if documenting his telephone number is insisted upon. It seems that he has taken many hospitals to court for suspicions about how his personal data has been handled.

Choose...

A 33 year old night security at a local hospital prefers to be alone whenever possible. He has no friends and he does not socialize. He does not keep update with current affairs and has no sexual interest. He spends most time daydreaming.

Choose...

Your answer is incorrect.

Explanation:

Patients with obsessive-compulsive personality disorder show difficulties in expressing warm or tender emotions to others, frequently they are perfectionists, often lacking clarity in seeing other perspectives or ways of doing things,. Rigid attention to detail may prevent them from completing tasks. Some may be hoarders, and often scrupulous with money. They may not be able to delegate tasks, and often get labeled as workaholics.

Patients with paranoid personality disorder are highly suspicious of other people and their motives, hold longstanding grudges against people, and believe others are not trustworthy. They are often emotionally detached and feel other people are deceiving, threatening, or making plans against them.

Patients with schizoid personality disorder have difficulties in expressing emotions, particularly around warmth or tenderness, prefer loneliness, appear aloof or remote, have difficulty in developing or maintaining social relationships, remain unaware of social trends, and are often unresponsive to praise or criticism.

The correct answer is: A 42-year-old woman has always been extremely neat and conscientious; she often stays long after normal working hours to check on punctuation errors. She had to do all jobs by herself and would not delegate her work. → Obsessive-compulsive personality disorder, A 35-year-old man refuses to provide answers to standard questions during an initial clerking and threatens to stop the interview if documenting his telephone number is insisted upon. It seems that he has taken many hospitals to court for suspicions about how his personal data has been handled. → Paranoid personality disorder, A 33 year old night security at a local hospital prefers to be alone whenever possible. He has no friends and he does not socialize. He does not keep update with current affairs and has no sexual interest. He spends most time daydreaming. → Schizoid personality disorder



Question 9

Not answered

Marked out of 3.00

Treatment of Affective Disorders

Choose the best option for each of the following clinical situations

Mr. X is a 40-year-old man with bipolar disorder currently depressed. He has been on Lithium for 5 years

Choose... ▾

Mrs. Y is a 25-year-old woman who has a history of bipolar disorder. She is now presenting with mixed affective state. She is also pregnant for the first time.

Choose... ▾

Mr. Z suffers from recurrent depression is being treated by his GP with venlafaxine. He has not made much improvement

Choose... ▾

Your answer is incorrect.

Explanation:

Q1: SSRIs should be tried as first line treatment option for depressive episodes in bipolar disorder.

Q2: Mixed affective states should be treated as a manic episode. Although she is pregnant, she will need treatment for bipolar disorder. Low dose of older first generation antipsychotics such as haloperidol could be used and have minimal risk of teratogenicity. There is little evidence that is available for the use of antipsychotics in pregnant woman with bipolar disorder.

Q3: In the management of recurrent depression with partial improvement, lithium should be tried first line drug as an augmentation strategy.

The correct answer is: Mr. X is a 40-year-old man with bipolar disorder currently depressed. He has been on Lithium for 5 years → Citalopram, Mrs. Y is a 25-year-old woman who has a history of bipolar disorder. She is now presenting with mixed affective state. She is also pregnant for the first time. → Haloperidol, Mr. Z suffers from recurrent depression is being treated by his GP with venlafaxine. He has not made much improvement → Lithium

Question 10

Not answered

Marked out of 5.00

Male: female ratios (1)

For each of the conditions given below choose male: female ratio from the drop-down list:

Anorexia nervosa

Choose... ▾

Major depression

Choose... ▾

Generalized anxiety disorder

Choose... ▾

Bipolar disorder type 1

Choose... ▾

Agoraphobia

Choose... ▾

Your answer is incorrect.

Explanation:

The male: female ratio for anorexia is 1:10.

In major depression, the male: female ratio is 1:2. This is the post-puberty prevalence rate. Before puberty, boys have marginally more depression than girls.

Generalized anxiety disorder is twice more often seen in females.

Bipolar disorder type 1 is a disorder with equal sex distribution of 1:1. In bipolar disorder type 2, the male: female ratio is 1:1.3.

Agoraphobia is more often seen in females with male: female ratio of 1:3.

The correct answer is: Anorexia nervosa → 1:10, Major depression → 1:2, Generalized anxiety disorder → 1:2, Bipolar disorder type 1 → 1:1, Agoraphobia → 1:3

Question 11

Not answered

Marked out of 4.00

Treatment of schizophrenia (2)

Choose one best management option for the following clinical situations

A 55-year-old man with chronic schizophrenia is experiencing troublesome auditory hallucinations not responding to any antipsychotic medication.

Choose... ▾

A 25-year-old man presents with paranoid delusions and auditory hallucinations that are derogatory in content. He has been tried on haloperidol and olanzapine for these symptoms but with minimal success.

Choose... ▾

A 45-year-old man is treated with olanzapine for persecutory delusions and auditory hallucinations for more than 10 months with some improvement of his psychotic symptoms. He now has low mood, difficulty enjoying himself, poor sleep and loss of appetite.

Choose... ▾

A 25-year-old man with schizophrenia has been treated on clozapine with good response to his positive symptoms. His social behavior is poor and there is a notable decline in his level of functioning.

Choose... ▾

Your answer is incorrect.

Explanation:

CBT should be tried in schizophrenia with treatment resistant delusions and hallucinations.

Clozapine should be tried at the earliest opportunity, if patients have been tried on 2 antipsychotics for adequate dosages and duration (Kane criteria).

The diagnosis is post schizophrenic depression and SSRIs should be tried as the first line treatment for patients with depression.

Cognitive remediation therapy should be given in schizophrenic patients with deterioration in function and social behaviour. Cognitive Remediation Therapy (CRT) is a cognitive rehabilitation therapy developed at King's College in London designed to improve neurocognitive abilities such as attention, working memory, cognitive flexibility and planning, and executive functioning which leads to improved social functioning. CRT has been used in the treatment of schizophrenia with positive results (See meta-analysis by Wykes et al, 2010 AJPsysh).

The correct answer is: A 55-year-old man with chronic schizophrenia is experiencing troublesome auditory hallucinations not responding to any antipsychotic medication. → CBT, A 25-year-old man presents with paranoid delusions and auditory hallucinations that are derogatory in content. He has been tried on haloperidol and olanzapine for these symptoms but with minimal success. → Clozapine, A 45-year-old man is treated with olanzapine for persecutory delusions and auditory hallucinations for more than 10 months with some improvement of his psychotic symptoms. He now has low mood, difficulty enjoying himself, poor sleep and loss of appetite. → SSRI, A 25-year-old man with schizophrenia has been treated on clozapine with good response to his positive symptoms. His social behavior is poor and there is a notable decline in his level of functioning. → Cognitive remediation therapy



Question 12

Not answered

Marked out of 3.00

Side effects of sexual nature

For each of the following side effects choose the most likely associated cause from the given list

A 34-year-old patient presents to emergency unit with painfully prolonged erection. He is being treated for low mood and insomnia.

Choose... ▾

A 41-year-old man complains of decreased sexual interest, as he is not able to ejaculate normally. He has been on a maintenance treatment for chronic auditory hallucinations.

Choose... ▾

A 32-year-old woman complains of new onset painful intercourse. She is recently being treated for long standing depression.

Choose... ▾

Your answer is incorrect.

Explanation: Priapism is reported in <1 in 1000 cases taking trazodone. From the given list of drugs, risperidone can cause dry ejaculation or retrograde ejaculation leading to loss of interest. Phenothiazines are worse offenders in this regard. Paroxetine produces anticholinergic dryness in vagina leading to vaginismus. Paroxetine is a worse offender when compared to other SSRIs in this regard.

The correct answer is: A 34-year-old patient presents to emergency unit with painfully prolonged erection. He is being treated for low mood and insomnia. → Trazodone, A 41-year-old man complains of decreased sexual interest, as he is not able to ejaculate normally. He has been on a maintenance treatment for chronic auditory hallucinations. → Risperidone, A 32-year-old woman complains of new onset painful intercourse. She is recently being treated for long standing depression. → Paroxetine

Question 13

Not answered

Marked out of 4.00

Diagnosis of anxiety disorders

Identify the diagnosis using the clinical descriptions given below;

A patient with long standing anxiety describes living on the edge all his life. He has recently deteriorated. He is performing poorly at work.

Choose... ▾

A 42-year-old mother with no previous psychiatric history presents with confusion and appears to be highly anxious. Her son developed an acute neurological illness and was hospitalized the previous day.

Choose... ▾

A 42-year-old woman is noted to be hoarding unnecessary things in large stock leading to concerns from her neighbours. She has never consulted a psychiatrist in the past.

Choose... ▾

A 45-year-old woman is noted to suffer from anxiety, depression, and poor concentration, irritability and is unable to work for last 4 weeks. She has divorced her husband recently.

Choose... ▾

Your answer is incorrect.

Explanation:

Generalized anxiety disorder is characterized by prominent tension, excessive worry with generalized free-floating persistent anxiety and feelings of apprehension about everyday events leading to significant stress and functional impairment. To diagnose GAD, ICD-10 requires duration of at least 6 months and the symptoms should have been present on most days during 6 months.

Acute stress reaction: starts usually in an hour; resolution starts within 8 hours if the stress is 'hit and run' and for 48 hours if it is prolonged. The stressor is usually one that poses serious threat to security, integrity and social position. The patient may initially be dazed with narrowed attention; disorientation is not uncommon as a result. Sometimes agitation and overactivity are seen.

Hoarding is often a feature of anankastic personality disorder. Rigid attention to detail may prevent them from completing tasks.

Adjustment disorder is a condition that refers to the psychological reactions arising in relation to adapting to new circumstances and occurs in someone who has been exposed to a psychosocial stressor like divorce, separation etc, which is not catastrophic in nature. The usual presentations include anxiety, depression, poor concentration, irritability, anger etc with physical symptoms caused by autonomic arousal

such as tremor and palpitations. The onset is more gradual than that of acute stress reaction and the course is more prolonged. Social functioning is usually impaired. Onset must be within 1 month in ICD and 3 months in DSM. Duration of adjustment disorder usually cannot exceed 6 months.

The correct answer is: A patient with long standing anxiety describes living on the edge all his life. He has recently deteriorated. He is performing poorly at work. → Generalised Anxiety Disorder, A 42-year-old mother with no previous psychiatric history presents with confusion and appears to be highly anxious. Her son developed an acute neurological illness and was hospitalized the previous day. → Acute stress reaction, A 42-year-old woman is noted to be hoarding unnecessary things in large stock leading to concerns from her neighbours. She has never consulted a psychiatrist in the past. → Anankastic personality disorder, A 45-year-old woman is noted to suffer from anxiety, depression, and poor concentration, irritability and is unable to work for last 4 weeks. She has divorced her husband recently. → Adjustment disorder



Question 14

Not answered

Marked out of 4.00

Rating scales (2)

Choose the most appropriate instrument for each of the following scenarios:

The scale used for obtaining information about a child's behaviour at school to aid in the diagnosis of ADHD

Choose...

A patient complains of multiple repetitive senseless thoughts. He also feels anxious as a result of this.

Choose...

A researcher wants to identify cases likely to have a mental disorder while conducting an epidemiological survey.

Choose...

This is a symptom inventory which measures symptoms of anxiety and depression

Choose...

Your answer is incorrect.

Explanation:

There are three Conners rating scales. One is designed for parents, another is for teachers, and a third Conners test asks adolescents to rate their own behavior. Completing Conners ADHD self-rated test takes from 5 to 30 minutes, depending on whether you're given the short or long version of the test. Long versions of the Conners ratings scales have about 60 to 90 questions, while short versions have less than 30 questions.

Yale Brown Obsessive Compulsive scale can be used to measure severity and assess common features of OCD.

GHQ is useful to identify cases likely to have a mental disorder.

The Hopkins symptoms checklist (HSCL-25) is a symptom inventory, which measures symptoms of anxiety and depression (not psychosis). It consists of 25 items: Part I of the HSCL-25 has 10 items for anxiety symptoms; Part II has 15 items for depression symptoms.

The correct answer is: The scale used for obtaining information about a child's behaviour at school to aid in the diagnosis of ADHD → Conners rating scale, A patient complains of multiple repetitive senseless thoughts. He also feels anxious as a result of this. → YBOCS, A

researcher wants to identify cases likely to have a mental disorder while conducting an epidemiological survey. → General Health Questionnaire, This is a symptom inventory which measures symptoms of anxiety and depression → Hopkins symptoms checklist (HSCL-25)

Question 15

Not answered

Marked out of 3.00

Compliance

For each of the clinical situations given below, identify the type of interventions helpful to ensure compliance

A 24-year-old male recently diagnosed with schizophrenia is currently on olanzapine. He is engaging well with nurses. However, there are concerns about his adherence to prescribed medications and risk of relapse.

Choose...

A 70-year-old lady with moderate cognitive impairment is living alone. You want to ensure she is taking her medication.

Choose...

A 35-year-old female is on citalopram for an anxiety disorder. Her adherence is erratic and she often takes extra tablets when she gets excessively worried. She lives with her husband.

Choose...

Your answer is incorrect.

Explanation: Depot injections would be an ideal choice if the patient is noncompliant with oral medications. In older people and in those with cognitive impairment, dosette box is a good way to ensure that he/she is taking their medications. If a patient lives with carer, supervised administration is ideal and carers/relatives can ensure that they are taking the medication.

The correct answer is: A 24-year-old male recently diagnosed with schizophrenia is currently on olanzapine. He is engaging well with nurses. However, there are concerns about his adherence to prescribed medications and risk of relapse. → Depot injection, A 70-year-old lady with moderate cognitive impairment is living alone. You want to ensure she is taking her medication. → Dosette box, A 35-year-old female is on citalopram for an anxiety disorder. Her adherence is erratic and she often takes extra tablets when she gets excessively worried. She lives with her husband. → Supervised administration

Question 16

Not answered

Marked out of 5.00

Side effects of medication

Match the side effects with the drugs most commonly associated with it

Thrombocytopenia	Choose... ▾
Gastric bleeding	Choose... ▾
Hypothyroidism	Choose... ▾
Cardiomyopathy	Choose... ▾
Urinary hesitancy	Choose... ▾

Your answer is incorrect.

Explanation:

The use of sodium valproate is associated with thrombocytopenia, leucopenia, red cell hypoplasia and pancreatitis. Thrombocytopenia is a dose related effect of valproate- decrease in dose is required if bruising, or bleeding gums is noted.

Several database-mining studies have found that patients who take SSRIs are at significantly increased risk of being admitted to hospital with an upper gastrointestinal bleeding compared with age and sex matched controls. The risk may be greatest with SSRIs that have a high affinity for the serotonin transporter. Lithium can cause variety of thyroid problems - the most common being a benign hypothyroid state. 5% patients may develop goiter and overt hyperthyroidism is also reported in some cases.

Thyroid deficiency is common in those with high risk for preexisting antithyroid antibodies such as middle-aged women. The risk is 3-4:1 in women and is high in first 2 years of treatment. Rapid cycling patients are at higher risk. High TSH is seen in nearly 1/3rd of chronic lithium treated patients - without clinical hypothyroidism. In resistant depression or non-responsive cyclers with bipolar it may be useful treating this subclinical state with thyroxine.

It has been suggested that clozapine is associated with myocarditis and cardiomyopathy. Myocarditis seems to occur within 6-8 weeks of starting clozapine and cardiomyopathy may occur later in treatment (median-9 months) but both may occur at any time.

Urinary hesitancy has been observed in around 10% of male patients taking part in the clinical-trials of reboxetine. Relief from this side effect could be achieved by using peripheral alpha1-receptor blockers tamsulosin or doxazosin.

The correct answer is: Thrombocytopenia → Sodium valproate, Gastric bleeding → Citalopram, Hypothyroidism → Lithium, Cardiomyopathy → Clozapine, Urinary hesitancy → Reboxetine



Question 17

Not answered

Marked out of 4.00

Neurotic Disorders

Please select one option from the list, which explains the following clinical situations.

A 24-year-old woman presents with feeling sad for three months, with loss of appetite. She doesn't want to do anything and has no motivation in his life.

Choose...

A 23-year-old woman complains of low mood for three years, chronic emptiness with history of multiple self-harm in the past. She was also sexually abused in the past.

Choose...

A 19-year-old college student presented with persecutory ideas, sadness, lack of concentration and perplexity. His academic performance has deteriorated recently and there is no evidence of hallucinations or thought disorder. He also complained of lack of interest and lack of appetite.

Choose...

A 32-year-old woman presented with excessive fear of being the focus of attention accompanied with a fear of vomiting when she attends family functions and public meetings

Choose...

Your answer is incorrect.

Explanation:

Case 1: This woman is presenting with biological and cognitive features of clinical depression.

Case 2: People with borderline personality disorder have fears of abandonment and display self-harming behaviour particularly during times of stress and crises. Often they have a history of sexual abuse in the past.

Case 3: People with early onset schizophrenia may not present with overt positive symptoms and may present with persecutory ideas initially. They may show decline in academic performance with poor concentration and poor motivation.

Case 4: Social phobia is characterised by symptoms of incapacitating anxiety, psychological and/or autonomic and are restricted to a particular social situations, leading to a desire for escape or avoidance. Individuals are often self-critical.

The correct answer is: A 24-year-old woman presents with feeling sad for three months, with loss of appetite. She doesn't want to do anything and has no motivation in his life. → Depressive episode, A 23-year-old woman complains of low mood for three years, chronic

emptiness with history of multiple self-harm in the past. She was also sexually abused in the past. → Borderline Personality Disorder, A 19-year-old college student presented with persecutory ideas, sadness, lack of concentration and perplexity. His academic performance has deteriorated recently and there is no evidence of hallucinations or thought disorder. He also complained of lack of interest and lack of appetite. → Schizophrenia, A 32-year-old woman presented with excessive fear of being the focus of attention accompanied with a fear of vomiting when she attends family functions and public meetings → Social phobia



Question 18

Not answered

Marked out of 5.00

Depot medications

Identify the most appropriate depot medication using the properties described below

May have an antidepressant effect

Choose...

Fewer incidence of extrapyramidal side effects

Choose...

May induce depressed mood

Choose...

Needs aqueous suspension to be constituted immediately before injection

Choose...

Test dose is often not required

Choose...

Your answer is incorrect.

Explanation:

Flupenthixol depot may have an antidepressant effect.

Pipotiazine depot has a fewer incidence of extrapyramidal side effects.

Fluphenazine depot can induce depressed mood.

Risperidone depot needs aqueous suspension immediately before injection and in practice, a test dose is not required.

The correct answer is: May have an antidepressant effect → Flupenthixol depot, Fewer incidence of extrapyramidal side effects → Pipotiazine depot, May induce depressed mood → Fluphenazine depot, Needs aqueous suspension to be constituted immediately before injection → Risperidone depot, Test dose is often not required → Risperidone depot

Question 19

Not answered

Marked out of 4.00

Subtypes of schizophrenia

Using the descriptions of some of the prominent symptoms given below, identify the type of schizophrenia from the list

This type is characterized by an early onset (usually in the second decade), very insidious and progressive course, and presence of characteristic negative symptoms.

Choose...

This type is characterized by marked thought disorder, severe loosening of associations, emotional disturbances characterized by inappropriate affect, blunted affect or senseless giggling, abnormal mannerisms like mirror gazing and markedly impaired social and occupational functioning

Choose...

This type is characterized by prominent delusions, which may or may not be persecutory in content and hallucinations, usually auditory. Thought disorders, affective abnormality and negative symptoms are seen but not dominant.

Choose...

This type has evidence of full-blown acute episode in the past but currently characterized only by negative symptoms.

Choose...

Your answer is incorrect.

Explanation:

Hebephrenic schizophrenia is characterised by marked thought disorder and severe loosening of associations, emotional disturbances characterised by inappropriate affect, blunted affect or senseless giggling, abnormal mannerisms like mirror gazing. There is markedly impaired social and occupational functioning; poor self care, poor hygiene, extreme social behaviour and disorganised behaviour.

Simple schizophrenia is characterised by an early onset (usually in the second decade), very insidious and progressive course, presence of characteristic negative symptoms like marked social withdrawal, loss of initiative and drive and shallow emotional response. People with this condition drift down the social ladder quickly, living shabbily and wandering aimlessly. Delusions and hallucinations are usually absent, if present they are short lasting and poorly systematised. The prognosis is usually very poor.

Residual schizophrenia consists of long term but not necessarily irreversible negative symptoms. Delusions and hallucinations must have been minimally intense or reduced for at least one-year period. The positive symptoms are gradually replaced by negative symptoms.

The correct answer is: This type is characterized by an early onset (usually in the second decade), very insidious and progressive course, and presence of characteristic negative symptoms. → Simple schizophrenia, This type is characterized by marked thought disorder, severe loosening of associations, emotional disturbances characterized by inappropriate affect, blunted affect or senseless giggling, abnormal mannerisms like mirror gazing and markedly impaired social and occupational functioning → Hebephrenic schizophrenia, This type is characterized by prominent delusions, which may or may not be persecutory in content and hallucinations, usually auditory. Thought disorders, affective abnormality and negative symptoms are seen but not dominant. → Paranoid schizophrenia, This type has evidence of full-blown acute episode in the past but currently characterized only by negative symptoms. → Residual schizophrenia



Question 20

Not answered

Marked out of 4.00

Subtypes of bipolar disorder

Select one diagnostic option for each of the following scenarios

A patient, who has been diagnosed with depression and was started on venlafaxine, now presents a history of feeling high with 'unstoppable energy'.

Choose...

A patient who is known to have mood disturbances is taking lithium. In the last 6 months he has had one episode each of low mood and mania, followed by another episode of high energy, sleep loss and euphoric mood. After an intervening period of normal mood, he developed another episode of mania.

Choose...

A 33-year-old man presents with a history of numerous periods with hypomanic symptoms and numerous periods with depressive symptoms in last 2 years that do not meet criteria for a mania or a major depressive episode.

Choose...

A 25-year-old woman has episodes of low mood and irritability. She described her energy levels to be high and is sleeping only for 4 hours a day.

Choose...

Your answer is incorrect.

Explanation:

Case 1 can be described as an example of bipolar 3 disorder; it is a variant of bipolar disorder with predominant depression complicated by antidepressant-induced hypomania. This presentation is a feature of the bipolar spectrum (in ICD: this is coded as unspecified type).

Case 2 is suggestive of rapid cycling, where at least four episodes of bipolar disorder occur within a period of one year. 70-80% of rapid cyclers are women. Some of the factors associated with the rapid cycling include the use of tricyclic antidepressant, low thyroxine level, being a female patient and having bipolar 2 rather than type 1 illness. Ultra-rapid cycling refers to the situation when fluctuations are over days or even hours.

Case 3 is suggestive of cyclothymia in which an individual may experience oscillating high and low moods (numerous periods of mild depression and mild elation), without ever having a manic or depressive episode that is sufficiently severe or prolonged to fulfil the criteria for BPAD or recurrent depressive disorder. The mood swings are usually as being unrelated to life events.

Mixed states are instances where manic and depressive symptoms occur simultaneously, as seen in case 4 above. The occurrence of both manic/hypo manic and depressive symptoms in a single episode, present every day for at least 1 week (DSM-5) or 2 weeks (ICD-10)

The correct answer is: A patient, who has been diagnosed with depression and was started on venlafaxine, now presents a history of feeling high with 'unstoppable energy'. → Bipolar disorder, unspecified, A patient who is known to have mood disturbances is taking lithium. In the last 6 months he has had one episode each of low mood and mania, followed by another episode of high energy, sleep loss and euphoric mood. After an intervening period of normal mood, he developed another episode of mania. → Rapid cycling bipolar disorder, A 33-year-old man presents with a history of numerous periods with hypomanic symptoms and numerous periods with depressive symptoms in last 2 years that do not meet criteria for a mania or a major depressive episode. → Cyclothymia, A 25-year-old woman has episodes of low mood and irritability. She described her energy levels to be high and is sleeping only for 4 hours a day. → Mixed affective state



Question 21

Not answered

Marked out of 5.00

Psychiatric symptoms in women

Identify the conditions specific to women, using the statements below

The onset of this disorder is within 1-2 weeks postpartum

Choose...

This disorder occurs in 10% of women postpartum

Choose...

This disorder may affect up to 40% of women of reproductive age.

Choose...

This condition peaks 3-4 days postpartum

Choose...

A woman with this condition firmly believes herself to be pregnant and develops objective signs in the absence of true pregnancy.

Choose...

Your answer is incorrect.

Explanation:

Post-partum psychosis generally starts within 1-2 weeks postpartum. Common features include lability of symptoms, insomnia, perplexity, disorientation, thoughts of suicide or infanticide.

Postnatal depression is a significant depressive episode occurring in 10-15% of women within 6 months post-partum (peak 3-4 weeks).

Pre-menstrual syndrome is a collection of psychological and somatic symptoms occurring during the luteal phase of menstrual cycle. This disorder may affect up to 40% of women of reproductive age. Symptoms may markedly interfere with work, school, social activity or relationships.

Postnatal blues (baby blues) is experienced by 3/4th of mothers who present with tearfulness and emotional lability starting 3-4 days after birth and lasting 1-2 days.

Pseudocyesis is a condition in which a woman firmly believes herself to be pregnant and develops objective pregnancy signs (abdominal enlargement, menstrual disturbance, apparent foetal movements, nausea, breast changes and labour pains) in the absence of pregnancy.

The correct answer is: The onset of this disorder is within 1-2 weeks postpartum → Postpartum psychosis, This disorder occurs in 10% of women postpartum → Postnatal depression, This disorder may affect up to 40% of women of reproductive age. → Premenstrual syndrome, This condition peaks 3-4 days postpartum → Postnatal blues, A woman with this condition firmly believes herself to be pregnant and develops objective signs in the absence of true pregnancy. → Pseudocyesis



Question 22

Not answered

Marked out of 5.00

Sleep disorders

Choose the most appropriate sleep disorder syndrome that applies to the descriptions given below

Vivid intense action packed violent dreams causing injury to self and partner. Some patients later develop movement disorders.

Choose...



Periodic somnolence and morbid hunger almost exclusively seen in males

Choose...



Unpleasant sensations in the legs particularly during the onset of sleep

Choose...



Repetitive episodes of upper airway obstruction during sleep, daytime somnolence, snoring and dry mouth.

Choose...



Cessation or decrease of ventilatory effort during sleep with intermittent snoring

Choose...



Your answer is incorrect.

Explanation:

REM sleep behavioural disorder is characterized by vivid intense action packed violent dreams, dream-enacting behaviours causing injury to self and partner and general sleep disruption. It is seen more commonly in older males with neurological disorders such as Parkinson's disease.

Kleine-Levin syndrome- Rare syndrome of periodic somnolence and morbid hunger, usually following a course of decreasing frequency of attacks, almost exclusively seen in male adolescents which may persist for many years.

Restless legs syndrome- Unpleasant often-painful sensations in the legs particularly on sleep onset, which significantly interfere with the ability to get to sleep. It is usually idiopathic, seen in elderly population.

Pickwickian syndrome (Obstructive sleep apnoea syndrome)- Repetitive episodes of upper airway obstruction during sleep, usually associated with reduced blood oxygen saturation, day time somnolence, snoring and dry mouth.



Central sleep apnoea syndrome is the cessation or decrease of ventilatory effort during sleep, more common in the elderly with degenerative neurological disorders. Intermittent snoring is present but not as loud as in obstructive sleep apnoea. It can lead to hypoxia and cardiac changes.

The correct answer is: Vivid intense action packed violent dreams causing injury to self and partner. Some patients later develop movement disorders. → REM sleep behavioural disorder, Periodic somnolence and morbid hunger almost exclusively seen in males → Kline-Levine syndrome, Unpleasant sensations in the legs particularly during the onset of sleep → Restless legs syndrome, Repetitive episodes of upper airway obstruction during sleep, daytime somnolence, snoring and dry mouth. → Pickwickian syndrome, Cessation or decrease of ventilatory effort during sleep with intermittent snoring → Central sleep apnoea syndrome



Question 23

Not answered

Marked out of 4.00

Disorders related to sexuality

Using the definitions given below, identify the most appropriate type of sexuality-related disorders from the attached list

A 45-year-old gentleman wears clothes of the opposite sex in order to experience temporary membership in the opposite sex. He has no desire for a permanent change to the opposite sex.

A 33-year-old gentleman expresses persistent discomfort with his sex. He feels like a woman and wants to be referred for a sex change surgery.

A 32-year-old gentleman wears his wife's undergarments and shoes. He obtains sexually satisfied by doing so and he does so to get the feeling of a woman.

A 46-year-old gentleman with moderate learning disability exposes his genitals to achieve sexual arousal

Your answer is incorrect.

Explanation:

Various degrees of **gender dysphoria** exist. One mild form is recognized in ICD and DSM as **dual role transvestism**. Here the individual wears clothes of the opposite sex in order to experience temporary membership in the opposite sex. The individual experiences a sense of appropriateness by wearing clothes of the other gender. There is no sexual motivation for the cross-dressing. The individual has no desire for a permanent change to the opposite sex. This must be differentiated from **fetishistic transvestism** where cross dressing results in sexual arousal often associated with masturbation or sexual activity. This is classified as a paraphilia. In **transsexualism** there is persistent discomfort with his/her sex or sense of inappropriateness in the gender role of the sex. There is strong and persistent cross-gender identification (not merely a desire for any perceived cultural advantages of being the other sex). This may be associated with wish to make one's body as congruent as possible with the preferred sex through surgery and hormone treatment. The disturbance is not concurrent with a physical intersex condition and not due to other functional psychiatric disorders. The disturbance causes clinically significant distress or impairment in social, occupational or other important areas of functioning. The transsexual identity has been present persistently for at least two years.

Exhibitionism is a type of paraphilia where an individual inappropriately exposes his/her genitals repeatedly to achieve sexual arousal.

The correct answer is: A 45-year-old gentleman wears clothes of the opposite sex in order to experience temporary membership in the opposite sex. He has no desire for a permanent change to the opposite sex. → Dual role transvestism, A 33-year-old gentleman expresses persistent discomfort with his sex. He feels like a woman and wants to be referred for a sex change surgery. → Transsexualism, A 32-year-old gentleman wears his wife's undergarments and shoes. He obtains sexually satisfied by doing so and he does so to get the feeling of a woman. → Fetishistic transvestism, A 46-year-old gentleman with moderate learning disability exposes his genitals to achieve sexual arousal → Exhibitionism



Question 24

Not answered

Marked out of 4.00

Somatoform and Dissociative disorders

For each of the following descriptions choose the most appropriate diagnosis from the list:

A 47-year-old woman presenting with a 4-year history of multiple vague physical symptoms not explained by any physical causes. She seeks repeated consultations with her GP s and hospital specialists.

Choose...

A 35-year-old gentleman who presents himself 30 miles away from his residence for no apparent reason with well-maintained self-care.

Choose...

A 43-year-old prisoner awaiting trial gives repeated wrong answers to questions, which are nonetheless 'in the right ballpark'

Choose...

A 40-year-old woman who is constantly pre-occupied that she has breast cancer despite all her relevant investigations were proved to be negative.

Choose...

Your answer is incorrect.

Explanation:

Somatisation is the experience of bodily symptoms with no, or no sufficient, physical cause for them, with presumed psychological causation. In hypochondriasis patient looks for diagnosis not symptom relief; he names suspected disorder and may be more or less convinced about having the disorder. In a patient with somatisation musculoskeletal symptoms are common; while in hypochondriasis gastrointestinal problems are the most common ones. Family history of alcohol use and antisocial personality may be more common in women with somatisation disorder.

Fugue is a dissociative reaction to unbearable stress. This could happen following a significant external stressor that the affected individual develops global amnesia and may wander to a distant location. Consciousness is unimpaired.

Ganser syndrome involves the production of approximate answers. Here the patient gives repeated wrong answers to questions, which are nonetheless in the right ballpark. Occasionally associated with organic brain illness, it is much more commonly seen as a form of malingering in those attempting to feign mental illness (e.g. in prisoners awaiting trial).

The correct answer is: A 47-year-old woman presenting with a 4-year history of multiple vague physical symptoms not explained by any physical causes. She seeks repeated consultations with her GP s and hospital specialists. → Somatisation disorder, A 35-year-old gentleman who presents himself 30 miles away from his residence for no apparent reason with well-maintained self-care. → Dissociative fugue, A 43-year-old prisoner awaiting trial gives repeated wrong answers to questions, which are nonetheless 'in the right ballpark' → Ganser's syndrome, A 40-year-old woman who is constantly pre-occupied that she has breast cancer despite all her relevant investigations were proved to be negative. → Hypochondriacal disorder



Question 25

Not answered

Marked out of 5.00

Sexual dysfunction

Identify the main causative factor, for each of the following conditions:

Vaginismus	Choose... 
Premature ejaculation	Choose... 
Priapism	Choose... 
Anorgasmia	Choose... 
Retrograde ejaculation	Choose... 

Your answer is incorrect.

Explanation: Vaginismus is characterized by an involuntary spasm of lower 1/3rd of vagina, interfering with coitus. Penile entry is painful or impossible. One of the important causative factors for vaginismus is traumatic early sexual experience in life.

Premature ejaculation is defined as an ejaculation before the completion of satisfactory sexual activity. The most common cause of premature ejaculation is performance anxiety (psychological).

Priapism (sustained penile erection) is a rare side effect of antidepressants like trazodone.

Fluoxetine is more likely to induce sexual side effects like anorgasmia.

Both delayed and retrograde ejaculations are reported with older antipsychotic drugs like thioridazine.

The correct answer is: Vaginismus → Traumatic early sexual life experiences, Premature ejaculation → Performance anxiety, Priapism → Trazadone, Anorgasmia → Fluoxetine, Retrograde ejaculation → Thioridazine

Question 26

Not answered

Marked out of 4.00

Assessment instruments

For each description below choose the most appropriate option from the list of scales:

A 34-year-old woman complains of loss of weight and appetite for last 2 months. She has poor sleep and has multiple somatic complaints that started when she and her husband separated 3 months ago. She denies using alcohol.

Choose...



A researcher is interested in finding out the degree of self-rated psychiatric symptoms in a sample of 125 diabetic outpatients attending an insulin clinic.

Choose...



As a part of a new World Mental Health initiative, it is planned to conduct a nationwide survey of mental health burden. It is planned to administer a fully structured instrument through trained non-clinical research workers to ascertain lifetime diagnosis of mental illness.

Choose...



A newly constituted mental health trust wants to adopt a common scale for monitoring clinical recovery on discharge across different adult units.

Choose...



Your answer is incorrect.

Explanation: Beck's depression scale (BDI) is a self-rated questionnaire containing 21 statements with four possible responses for each. The General health questionnaire (GHQ) is a self-rated screening instrument for the presence of psychiatric illness. The diagnostic interview schedule (DIS) is a non-clinician administered fully structured interview and used in Epidemiological catchment area study. The Health of Nations Outcome Scale is used for monitoring clinical recovery and is helpful to measure clinical outcomes in mental health.

The correct answer is: A 34-year-old woman complains of loss of weight and appetite for last 2 months. She has poor sleep and has multiple somatic complaints that started when she and her husband separated 3 months ago. She denies using alcohol. → Beck's depression scale, A researcher is interested in finding out the degree of self-rated psychiatric symptoms in a sample of 125 diabetic outpatients attending an insulin clinic. → General Health Questionnaire, As a part of a new World Mental Health initiative, it is planned to conduct a nationwide survey of mental health burden. It is planned to administer a fully structured instrument through trained non-clinical research workers to ascertain

lifetime diagnosis of mental illness. → Diagnostic Interview Scale, A newly constituted mental health trust wants to adopt a common scale for monitoring clinical recovery on discharge across different adult units. → Health of Nations Outcome Scale

Question 27

Not answered

Marked out of 5.00

Minimum effective dose in first-episode of schizophrenia

Choose the minimum antipsychotic dose likely to be effective in first episode schizophrenia for each of the following drugs

Olanzapine	Choose... ▾
Risperidone	Choose... ▾
Quetiapine	Choose... ▾
Amisulpride	Choose... ▾
Chlorpromazine	Choose... ▾

Your answer is incorrect.

Explanation: The minimum dose of antipsychotics likely to be effective in first episode schizophrenia include Olanzapine- 5mg, Risperidone- 1-2 mg, Quetiapine- 150 mg, Amisulpride-150 mg, Sulpiride- 150 mg, Haloperidol- 2mg, Chlorpromazine-200 mg.

Please note for treating a relapse of schizophrenia, the doses required are generally higher and are as follows; Olanzapine- 10mg, Risperidone- 3-4 mg, Quetiapine- 300 mg, Amisulpride-800 mg, Sulpiride- 800 mg, Haloperidol- > 4mg, Chlorpromazine-300 mg. (Maudsley Prescribing Guidelines)

The correct answer is: Olanzapine → 5 mg, Risperidone → 1-2 mg, Quetiapine → 150 mg, Amisulpride → 400 mg, Chlorpromazine → 200 mg

Question 28

Not answered

Marked out of 5.00

Clozapine related adverse effects

For each of the following side effects caused by clozapine, identify the best plan of action

A 34-year-old woman started on clozapine suffers from high temperature

Choose...

A 65-year old gentleman with one episode of seizures 10 years ago now requires high dose of clozapine to treat psychosis

Choose...

A 33-year-old woman has gained weight on clozapine after taking it for 3 months

Choose...

A 45-year-old woman complains of excessive salivation especially at night time

Choose...

A 56-year-old woman recently started on clozapine complains of dizzy spells on waking up from bed

Choose...

Your answer is incorrect.

Explanation:

Fever is common in the first 3 weeks after starting on clozapine treatment. Clozapine induces inflammatory response (increased C- reactive protein and interleukin 6). The first step is to give antipyretics and check full blood count.

Seizures are dose related side effects of clozapine treatment. Consider prophylactic valproate if on high dose or with high plasma levels. EEG abnormalities are common in those on clozapine.

For clozapine induced weight gain, dietary counselling is essential.

Hypersalivation due to clozapine treatment is very troublesome at night and last for many months. Hyoscine 300 micrograms sucked and swallowed at night may be an option though not much evidence base exists in this regard. Pirenzepine can also be tried.

Hypotension usually occurs in the early phase of treatment (first 4 weeks). It is important to advise patient to take time when standing up. If hypotension occurs, reduce dose or slow rate of increase. If very severe, consider fludrocortisone.

The correct answer is: A 34-year-old woman started on clozapine suffers from high temperature → Check full blood count, A 65-year old gentleman with one episode of seizures 10 years ago now requires high dose of clozapine to treat psychosis → Consider Prophylactic valproate treatment, A 33-year-old woman has gained weight on clozapine after taking it for 3 months → Dietary counselling, A 45-year-old woman complains of excessive salivation especially at night time → Hyoscine 300 micrograms sucked and swallowed at night time, A 56-year-old woman recently started on clozapine complains of dizzy spells on waking up from bed → Advise patients to change posture gradually



Question 29

Not answered

Marked out of 3.00

Adverse effects (2)

Identify the type of side effects, using descriptions given below;

A 36-year-old woman treated on fluoxetine and phenelzine for her depression has now developed diarrhoea, excessive sweating and myoclonus.

Choose...

A 76-year-old gentleman admitted to hospital is on paroxetine for more than 5 years. He has not taken his antidepressant medication for last 5 days. He has developed flu like symptoms, paresthesias and complains of electric shock like sensations

Choose...

A 73-year-old woman started on fluoxetine recently was noticed by her CPN to be confused and is complaining of dizziness, nausea and lethargy

Choose...

Your answer is incorrect.

Explanation:

Serotonin syndrome is a result of excessive serotonergic transmission in brain. Although no single receptor appears to be responsible, it is possibly mediated via 5HT 2A receptor stimulation. Diarrhea, myoclonus, diaphoresis, hyperactive reflexes, ataxia, hypomanic or labile mood, tremors and disorientation are the common features. It may mimic NMS or anticholinergic syndrome in those receiving psychotropics.

The abrupt withdrawal of SSRI especially paroxetine, fluvoxamine or venlafaxine (shorter half-life), is associated with dizziness, nausea, headache, depression, anxiety, insomnia, poor concentration, flu-like symptoms, paresthesias, and migraine-like symptoms. It usually requires at least 6 weeks of treatment before discontinuation and resolves spontaneously in 3 weeks.

Most antidepressants are associated with hyponatraemia. The onset is usually within 30 days of treatment and is probably not dose-related. The mechanism of this adverse effect is likely to be mediated via the syndrome of inappropriate ADH secretion (SIADH). It has been suggested that serotonergic drugs are relatively more likely to cause hyponatraemia. The clinical symptoms include dizziness, lethargy, nausea, confusion, cramps and seizures.

The correct answer is: A 36-year-old woman treated on fluoxetine and phenelzine for her depression has now developed diarrhoea, excessive sweating and myoclonus. → Serotonin syndrome, A 76-year-old gentleman admitted to hospital is on paroxetine for more than 5 years. He has not taken his antidepressant medication for last 5 days. He has developed flu like symptoms, paresthesias and complains of electric shock like sensations → Antidepressant discontinuation syndrome, A 73-year-old woman started on fluoxetine recently was noticed by her CPN to be confused and is complaining of dizziness, nausea and lethargy → Hyponatraemia

Question 30

Not answered

Marked out of 3.00

Extrapyramidal side effects

Choose the most appropriate scale to quantify and/or examine the extrapyramidal side effects described below

A 66-year-old lady on depot zuclopenthixol presents with tremor, rigidity and bradykinesia

Choose... 

A 34-year-old man treated on risperidone is constantly pacing up and down. He complains of an unpleasant state of restlessness

Choose... 

A 67-year-old woman on depot flupenthixol presents with lip smacking and pill-rolling hand movements

Choose... 



Your answer is incorrect.

Explanation: The diagnosis is Pseudo-parkinsonism and the scale used is Simpson-Angus EPS rating scale. The diagnosis is Akathisia and the scale used is Barnes akathisia scale. The diagnosis is Tardive dyskinesia and the scale used is abnormal involuntary movements scale (AIMS).

The correct answer is: A 66-year-old lady on depot zuclopenthixol presents with tremor, rigidity and bradykinesia → Simpson-Angus EPS rating scale, A 34-year-old man treated on risperidone is constantly pacing up and down. He complains of an unpleasant state of restlessness → Barnes akathisia scale, A 67-year-old woman on depot flupenthixol presents with lip smacking and pill-rolling hand movements → Abnormal involuntary movement scale

Question 31

Not answered

Marked out of 4.00

Movement disorders

Which one of the listed options is the most likely diagnosis for the following patients?

A 23-year-old woman describes episodes of sudden collapse to the ground following laughing and excitement.

Choose... ▾

A 22-year-old man was given high dose of haloperidol intramuscularly for his psychotic symptoms and severe agitation. After few hours, he complains of painful involuntary protrusion of his tongue

Choose... ▾

A 79-year-old man was admitted to the neurology ward for left sided hemiplegia due to haemorrhagic stroke. He develops sudden purposeless vigorous 'throwing movements' of his arm and trunk

Choose... ▾

A 64-year-old man with rapid onset dementia develops brief muscular jerks of the extremities

Choose... ▾

Your answer is incorrect.

Explanation:

Cataplexy is a symptom of narcolepsy in which there is a sudden loss of muscle tone leading to collapse. Cataplexy usually occurs following emotional stress.

Dystonia is a side effect of treatment with antipsychotic medications. Dystonias are brief or prolonged contractions of specific groups of muscles resulting in symptoms such as oculogyric crises, tongue protrusion, torticollis and blepharospasm.

Hemiballismus is involuntary large scale 'throwing' movements of one limb or one body side.

Case 4: This patient is suffering from Creutzfeldt-Jakob disease, which is characterised by rapid onset dementia, and myoclonus (a brief involuntary twitching of a muscle or group of muscles). The myoclonic twitches can be brought on by sudden muscular contractions.

The correct answer is: A 23-year-old woman describes episodes of sudden collapse to the ground following laughing and excitement. → Cataplexy, A 22-year-old man was given high dose of haloperidol intramuscularly for his psychotic symptoms and severe agitation. After few hours, he complains of painful involuntary protrusion of his tongue → Dystonia, A 79-year-old man was admitted to the neurology ward for

left sided hemiplegia due to haemorrhagic stroke. He develops sudden purposeless vigorous 'throwing movements' of his arm and trunk → Hemiballismus, A 64-year-old man with rapid onset dementia develops brief muscular jerks of the extremities → Myoclonus



Question 32

Not answered

Marked out of 4.00

Serious Adverse Effects

Select the most possible diagnosis for each of the following clinical situations;

A 67-year-old woman has a history of recurrent bipolar depression. She has been commenced on Lamotrigine. Within a week she has developed fever, extremely painful mouth ulcers and small round lesions in the face and body.

Choose...

A 45-year-old woman with a history of bipolar disorder is on lithium carbonate for more than 2 years. She is complaining of weight gain, constipation and tiredness

Choose...

An 82-year-old woman has been diagnosed with urinary tract infection. She has high fever and is dehydrated. She is on lithium carbonate and now aspirin. She has developed vomiting, unsteadiness of gait and coarse tremors.

Choose...

A 49-year-old man with chronic schizophrenia on clozapine treatment. He complains of sweating, palpitations and breathing difficulties.

Choose...

Your answer is incorrect.

Explanation:

Q1: This is Stevens-Johnson syndrome, caused by Lamotrigine. SJS usually begins with fever, sore throat, and fatigue, which is misdiagnosed and usually treated with antibiotics. Ulcers and other lesions begin to appear in the mucous membranes, almost always in the mouth and lips but also in the genital and anal regions. Those in the mouth are usually extremely painful and reduce the patient's ability to eat or drink. A rash of round lesions about an inch across arises on the face, trunk, arms and legs, and soles of the feet, but usually not the scalp.

Q2: Lithium induced hypothyroidism occurs more commonly in females (M:F ratio is 1:3-4). It is common in those with high risk of preexisting thyroid antibodies such as middle aged woman.

Q3: Lithium toxicity occurs in conditions of overdose or dehydration. Gastrointestinal symptoms are followed by neurological symptoms. Fine tremor is a common benign side effect of lithium but coarse tremor is a sign of toxicity.

Q4: The diagnosis is clozapine induced cardiomyopathy, which may occur late in treatment with clozapine (median-9 months).

The correct answer is: A 67-year-old woman has a history of recurrent bipolar depression. She has been commenced on Lamotrigine. Within a week she has developed fever, extremely painful mouth ulcers and small round lesions in the face and body. → Stevens-Johnson syndrome, A 45-year-old woman with a history of bipolar disorder is on lithium carbonate for more than 2 years. She is complaining of weight gain, constipation and tiredness → Hypothyroidism, An 82-year-old woman has been diagnosed with urinary tract infection. She has high fever and is dehydrated. She is on lithium carbonate and now aspirin. She has developed vomiting, unsteadiness of gait and coarse tremors. → Lithium toxicity, A 49-year-old man with chronic schizophrenia on clozapine treatment. He complains of sweating, palpitations and breathing difficulties. → Cardiomyopathy



Question 33

Not answered

Marked out of 4.00

Neurotic Disorders (2)

Please select one option from the list, which explains the following clinical situations.

A 68-year-old gentleman was admitted to the medical ward with history of chest pain and no medical cause was found. He told that about 10 days ago, his house was attacked and he lost many things. He progressed well in the ward, appeared cheerful and was optimistic of discharge and the future but refused to go back to his home. His relatives informed you that his pre-morbid functioning was stable.

Choose...

A 53-year-old woman lost her son in a car accident while she was the passenger. 4 months later she started to experience intrusive thoughts about the incident and began to avoid going to any place resembling it. She also had repeated flashbacks of the event and she complained of poor concentration and feeling anxious.

Choose...

A 31-year-old woman when at home, witnessed her husband developing seizure and she dropped her 2-year-old son at that time to the floor. Later her son was stabilized in the paediatric ward and her husband was recovering from A&E where she was noticed to be trembling, sweating, very anxious and also complained of difficulty in swallowing. When approached by A&E staff, she was very agitated and abusive towards them.

Choose...

A 35-year-old woman has been frightened to go out of her house and prefers to stay at home, when she has to go out for appointments, she usually can't sleep the night before, fears as if she is going to die before leaving and usually returns back soon after leaving home without attending them. Recently she started to abuse alcohol.

Choose...

Your answer is incorrect.

Explanation:

Acute stress reaction: starts usually in an hour; resolution starts within 8 hours if the stress is hit and run for 48 hours if it is prolonged. Presence of physical exhaustion, organic factors or disease states increases the risk. The stressor is usually one that poses serious threat to security, integrity and social position. The patient may initially be dazed with narrowed attention; disorientation is not uncommon as a result. Sometimes agitation and overactivity are seen. Partial or complete amnesia for the acute stress reaction is not unheard of. Dissociative symptoms seem to predominate in some.

PTSD should start within 6 months of the trauma. In a small number of patients the onset is delayed i.e. after 6 months - termed as 'probable TSD'; in others the course may be chronic > 6 months.

Agoraphobia is characterised by symptoms of anxiety (psychological and/or autonomic) and are restricted to places or situations where escape may be difficult or embarrassing, leading to avoidance (E.g. of crowds, public places, travelling away from home or alone).

The correct answer is: A 68-year-old gentleman was admitted to the medical ward with history of chest pain and no medical cause was found. He told that about 10 days ago, his house was attacked and he lost many things. He progressed well in the ward, appeared cheerful and was optimistic of discharge and the future but refused to go back to his home. His relatives informed you that his pre-morbid functioning was stable. → Normal adaptive reaction, A 53-year-old woman lost her son in a car accident while she was the passenger. 4 months later she started to experience intrusive thoughts about the incident and began to avoid going to any place resembling it. She also had repeated flashbacks of the event and she complained of poor concentration and feeling anxious. → Post traumatic stress disorder, A 31-year-old woman when at home, witnessed her husband developing seizure and she dropped her 2-year-old son at that time to the floor. Later her son was stabilized in the paediatric ward and her husband was recovering from A&E where she was noticed to be trembling, sweating, very anxious and also complained of difficulty in swallowing. When approached by A&E staff, she was very agitated and abusive towards them. → Acute stress reaction, A 35-year-old woman has been frightened to go out of her house and prefers to stay at home, when she has to go out for appointments, she usually can't sleep the night before, fears as if she is going to die before leaving and usually returns back soon after leaving home without attending them. Recently she started to abuse alcohol. → Agoraphobia with panic disorder



Question 34

Not answered

Marked out of 5.00

Treatment of depression

Choose the best drug for the following clinical situations

A 33-year-old woman complains of low mood, anhedonia and low energy levels present for more than 2 months

Choose... ▾

A 43-year-old woman has been treated on SSRIs for depression. She has severe insomnia, which has not responded to treatment. She is not keen on hypnotics

Choose... ▾

A 66-year-old woman is suffering from both severe depression and anxiety. She has been on SSRIs with no benefit for 3 months

Choose... ▾

A 45-year-old man with history of depression suffers from sexual side effects of paroxetine. He has a history of hypertension

Choose... ▾

A 75-year-old man has been diagnosed with severe depression for the first time following the death of his wife.

Choose... ▾

Your answer is incorrect.

Explanation: For mild/moderate depression, **SSRIs** such as citalopram should be used as first line treatment. **Trazodone** could be used in patients with depression and insomnia. Some combinations may be used for specific symptom relief e.g. trazodone for insomnia in combination with other antidepressants such as SSRIs. **Venlafaxine** is helpful in the treatment of depression and anxiety. **Mirtazapine** is a dual acting agent, which does not induce sexual side effects and could be safely used in people with hypertension. In old age depression, SSRIs should be tried as first line treatment in the treatment of depression

The correct answer is: A 33-year-old woman complains of low mood, anhedonia and low energy levels present for more than 2 months → Citalopram, A 43-year-old woman has been treated on SSRIs for depression. She has severe insomnia, which has not responded to treatment. She is not keen on hypnotics → Trazadone, A 66-year-old woman is suffering from both severe depression and anxiety. She has been on SSRIs with no benefit for 3 months → Venlafaxine, A 45-year-old man with history of depression suffers from sexual side effects of paroxetine. He has a history of hypertension → Mirtazapine, A 75-year-old man has been diagnosed with severe depression for the first time following the death of his wife. → Citalopram

Question 35

Not answered

Marked out of 4.00

Emergency Psychiatry

For each of the following situations, identify the most suitable diagnosis;

A 56-year-old man presented to A& E with a history of falls and sustained hip fracture. Following surgery he is now presenting with confusion and visual hallucinations. On examination he has marked tremor in both hands. His wife described him to be an alcoholic.

Choose...

A 66-year-old woman on carbamazepine was admitted to medical ward with nausea, lethargy, dizziness and confusion. Her serum sodium level was 126 mmol/L

Choose...

A 73-year-old man was brought to A& E with weakness involving his right arm. His speech is noted to be non-fluent and he has a mild level of cognitive impairment

Choose...

A 55-year-old man complains of severe headaches and sickness in the morning. He has memory problems, difficulty in understanding instructions and reading printed text.

Choose...

Your answer is incorrect.

Explanation:

Delirium tremens is a toxic confusional state that occurs when alcohol withdrawal symptoms are severe. The classic triad of symptoms include clouding of consciousness & confusion, vivid hallucinations affecting every sensory modality and marked tremor.

Carbamazepine has a well known association with hyponatraemia. The mechanism of this adverse effect is syndrome of inappropriate ADH secretion (SIADH). The patients usually presents with signs of hyponatraemia such as nausea, lethargy, dizziness, confusion, cramps and seizures.

Left frontal lobe infarct involves the Broca's area of the brain. Broca's aphasia from a lesion in areas 44 and 45 on the left hemisphere leads to nonfluent speech, agrammatism, paraphasias, anomia, and poor repetition.

Tumours of the parietal lobe involve the language centre (Wernicke's area), leading to difficulty talking and reading and difficulty understanding what is being said. Brain tumours can also lead to memory loss. Headaches and sickness are quite severe and prolonged and are worst in the morning.

The correct answer is: A 56-year-old man presented to A& E with a history of falls and sustained hip fracture. Following surgery he is now presenting with confusion and visual hallucinations. On examination he has marked tremor in both hands. His wife described him to be an alcoholic. → Delirium tremens, A 66-year-old woman on carbamazepine was admitted to medical ward with nausea, lethargy, dizziness and confusion. Her serum sodium level was 126 mmol/L → Inappropriate ADH secretion, A 73-year-old man was brought to A& E with weakness involving his right arm. His speech is noted to be non-fluent and he has a mild level of cognitive impairment → Frontal lobe infarct, A 55-year-old man complains of severe headaches and sickness in the morning. He has memory problems, difficulty in understanding instructions and reading printed text. → Parietal lobe tumour



Question 36

Not answered

Marked out of 5.00

Management of anxiety disorders

Choose the first line drug treatments and the diagnosis for each of the following conditions

A 56 year old woman with irrational worries, motor tension, hypervigilance and somatic symptoms such as sweating and tachycardia

Choose...



A 23 year old woman with sudden unpredictable episodes of severe anxiety, shortness of breath, fear of dying and urgent desire to flee

Choose...



A 27-year-old woman has suffered from a car accident when she drove it and lost her husband. She has vivid dreams, flashbacks and is avoiding discussing anything related to the accident

Choose...



A 33 year old woman who is constantly thinking that the door has been left unlocked and is constantly going back to check

Choose...



A 29-year-old man with extreme fear of eating in public spaces due to fear of embarrassment. He never eats in a restaurant and is feeling sick on even entering a restaurant

Choose...



Your answer is incorrect.

Explanation:

Q1: The diagnosis is generalised anxiety disorder. SSRIs should be used as first line drug treatment although it may initially exacerbate symptoms. A lower starting dose is often required.

Q2: The diagnosis is panic disorder. The therapeutic effect of SSRIs can be delayed, although panic symptoms can return quickly if the drug is withdrawn.

Q3: The diagnosis is post traumatic stress disorder and SSRIs should be used.

Q4: The diagnosis is obsessive compulsive disorder characterised by obsessional thinking (constantly thinking that the door has been left unlocked) and compulsive behaviour (constantly going back to check). SSRIs are the first line drug treatments for the treatment of OCD.

Q5: The diagnosis is social phobia characterised by extreme fear of social situations, fear of humiliation and embarrassment, avoidant behaviour and anxious anticipation. SSRIs should be used as first line drug treatments.

The correct answer is: A 56 year old woman with irrational worries, motor tension, hypervigilance and somatic symptoms such as sweating and tachycardia → Generalised anxiety disorder, SSRIs, A 23 year old woman with sudden unpredictable episodes of severe anxiety, shortness of breath, fear of dying and urgent desire to flee → Panic disorder, SSRIs, A 27-year-old woman has suffered from a car accident when she drove it and lost her husband. She has vivid dreams, flashbacks and is avoiding discussing anything related to the accident → PTSD, SSRIs, A 33 year old woman who is constantly thinking that the door has been left unlocked and is constantly going back to check → OCD, SSRIs, A 29-year-old man with extreme fear of eating in public spaces due to fear of embarrassment. He never eats in a restaurant and is feeling sick on even entering a restaurant → Social Phobia, SSRIs



Question 37

Not answered

Marked out of 3.00

Teratogenicity

Choose one most appropriate option for each question below:

A 30-year-old woman suffering from bipolar affective disorder has been on remission for a long time on lithium. She has a family history of Down's syndrome. What is the most important adverse effect you will warn her about if she is planning to become pregnant?

Choose...



A 33-year-old woman suffering from hypomanic episode. She has a long history of bipolar affective disorder. After improvement she is now maintained on sodium valproate. What is the most important adverse effect you will warn her about when she is planning to become pregnant?

Choose...



A 27-year-old woman, who is 5 months pregnant now, is now depressed. She has a past and a family history of depression. She had a good response to paroxetine and insists to continue paroxetine during this episode. What adverse effect will you warn her about?

Choose...



Your answer is incorrect.

Explanation:

Lithium- 1 in 10 chance of having a malformation if lithium is continued through first trimester. Ebstein's anomaly-Relative risk compared to general population-10-20times higher, but the absolute risk is low at 1:1000. (Absolute spontaneous risk of Ebstein's is 1 in 20,000. Cohen et al., JAMA 1994;271:146-150) Maximum risk is at 2-6 weeks after conception when many pregnancies are still undiscovered.

Sodium valproate is the most teratogenic of all psychotropics. Risk for any birth defect quoted 7.2% in Maudsley & NICE. The risk is dose related and mostly seen in 17 to 30 days post conception; risk increases with family history of neural defects. The congenital anomalies associated with sodium valproate include neural tube defects (1-2 %), especially spina bifida (10 fold increase), digital and limb defects.

Paroxetine, particularly high dose first trimester exposure, is clearly linked to cardiac malformation - VSD and ASD. Third trimester use can give rise to neonatal complication due to abrupt withdrawals.

The correct answer is: A 30-year-old woman suffering from bipolar affective disorder has been on remission for a long time on lithium. She has a family history of Down's syndrome. What is the most important adverse effect you will warn her about if she is planning to become



pregnant? → Ebstein's anomaly, A 33-year-old woman suffering from hypomanic episode. She has a long history of bipolar affective disorder. After improvement she is now maintained on sodium valproate. What is the most important adverse effect you will warn her about when she is planning to become pregnant? → Neural tube defects, A 27-year-old woman, who is 5 months pregnant now, is now depressed. She has a past and a family history of depression. She had a good response to paroxetine and insists to continue paroxetine during this episode. What adverse effect will you warn her about? → Cardiac malformations



Question 38

Not answered

Marked out of 5.00

Poisoning & psychiatric presentations

You are the liaison psychiatrist on-call. Five patients have presented to A&E department with history of poisoning. For each of the presentations given below, identify the most likely diagnosis from list below

A 45-year-old man with history of bloody stools, diarrhoea and vomiting. He appears restless, anxious and is unable to concentrate.

Choose... ▾

A 55-year-old man with loss of appetite and colicky pain. He also has cognitive impairment

Choose... ▾

A 49-year-old man with headache, irritability, joint pains and somnolence. She has tremors, rigidity, mask like face and dysarthria

Choose... ▾

A 63-year-old woman has vomiting and diarrhoea. She complains of hair loss and severe pain in the legs. She is paranoid and confused.

Choose... ▾

A 55-year-old man has worked in a pesticide factory for more than 20 years. He has poor concentration, recent memory impairment and impairment of new learning

Choose... ▾

Your answer is incorrect.

Explanation:

Mercury poisoning is featured by CNS symptoms of lethargy and restlessness. The primary symptoms are secondary to severe gastrointestinal irritation, with bloody stools, diarrhoea and vomiting leading to circulatory collapse. It causes depression and mild cognitive impairment with poor attention and concentration with severe anxiety and agitation (Mad hatter's disease).

Lead poisoning in children causes an encephalopathy that may be associated with permanent intellectual impairment. Adults exhibit anorexia and colic with cognitive impairment and with chronic exposure can cause psychosis.

Manganese poisoning produces symptoms such as headache, irritability, joint pains and somnolence (manganese madness). Lesions involving basal ganglia and pyramidal system result in gait impairment, rigidity, monotonous or whispering speech, tremor of extremities and tongue, masked facies, micrographia, dystonia, dysarthria and loss of equilibrium.

Thallium poisoning causes severe pain in the legs as well as diarrhoea and vomiting. Within a week people may develop delirium, convulsions, cranial nerve palsies, blindness, choreiform movements and coma may occur. Alopecia is a common and important diagnostic clue. Behavioral changes include paranoid thinking and depression, with suicidal tendencies. Thallium poisoning almost always follows accidental or intentional ingestion of a rat poison 5)

Arsenic poisoning most often occurs during industrial exposure, especially workers in production of pesticides, copper or lead smelting. Psychiatric symptoms include mild impairment of new learning, recent memory and concentration. This may manifest a few years after exposure. Recovery is possible on cessation of the exposure.

The correct answer is: A 45-year-old man with history of bloody stools, diarrhoea and vomiting. He appears restless, anxious and is unable to concentrate. → Mercury poisoning, A 55-year-old man with loss of appetite and colicky pain. He also has cognitive impairment → Lead poisoning, A 49-year-old man with headache, irritability, joint pains and somnolence. She has tremors, rigidity, mask like face and dysarthria → Manganese poisoning, A 63-year-old woman has vomiting and diarrhoea. She complains of hair loss and severe pain in the legs. She is paranoid and confused. → Thallium poisoning, A 55-year-old man has worked in a pesticide factory for more than 20 years. He has poor concentration, recent memory impairment and impairment of new learning → Arsenic poisoning



Question 39

Not answered

Marked out of 4.00

Congenital abnormalities & medications

Select the most likely drug associated with the congenital abnormalities described below;

Persistent pulmonary hypertension	Choose... ▾
Ebstein's anomaly	Choose... ▾
Mental retardation	Choose... ▾
Hypospadias	Choose... ▾

Your answer is incorrect.

Explanation:

Paroxetine, particularly high dose first trimesters exposure, is clearly linked to cardiac malformation - VSD and ASD. Third trimester use can give rise to neonatal complication due to abrupt withdrawals. Exposure to SSRI, when taken in late pregnancy, may increase risk of persistent pulmonary hypertension of the newborn, although the absolute risk is small.

Lithium has been associated with an increased risk (1:1000) of Ebstein's anomaly (downward displacement of the tricuspid valve into the third ventricle) and detailed ultrasonography/fetal echocardiography is indicated at 16-18 weeks.

It is increasingly recognised that low IQ in children is the most common result of mother being treated with valproate when pregnant. Among children whose mothers took valproate monotherapy in pregnancy 42% had a verbal IQ score of less than 80 (22%, <70 Adab et al, 2004). Valproate has also been associated with increased risk of hypospadias.

The correct answer is: Persistent pulmonary hypertension → Paroxetine, Ebstein's anomaly → Lithium, Mental retardation → Sodium valproate, Hypospadias → Sodium valproate

Question 40

Not answered

Marked out of 3.00

Breast feeding & psychotropic medication (2)

Choose the most appropriate drug for each of the following situations;

A 33-year-old woman delivered a female baby 4 months ago and is breast-feeding. She has now been diagnosed with bipolar disorder. You have decided to start her on a mood stabilizer. Which one should be avoided in this case?

Choose...

A 29-year-old mother who is currently nursing her 6-month-old child has now been diagnosed with depression. You are keen to start her on SSRIs for her depression

Choose...

A 27-year-old breast-feeding mother has been diagnosed with depression. She has had depression in the past and responded to one of the older antidepressants. She insists trying one of the older drugs again.

Choose...

Your answer is incorrect.

Explanation:

Lithium is contraindicated during breast-feeding (Kohen, 2005). Serum concentration and blood count should be monitored for infants exposed to lithium. 40- 50% of maternal serum level is excreted in breast milk. Infant serum level can rise up to 200% of maternal serum concentration (range 5%-200%). Serum level of lithium can be raised due to diminished renal clearance in neonates.

SSRI and tricyclic antidepressant (except Doxepin) are generally safe. Paroxetine has a lower milk/plasma ratio than fluoxetine and sertraline. Fluoxetine and its active metabolite norfluoxetine were detected in plasma and breast milk but not in infant's plasma.

The preferred tricyclic antidepressants for use in nursing mothers are Amitriptyline and Imipramine. Clomipramine and nortriptyline also do not show any adverse effects in infants. Maternal plasma and breast milk concentration of tricyclic antidepressant is the same. N-desmethyldoxepine, longer acting metabolite of doxepin, may accumulate in infants and cause severe drowsiness and respiratory depression.

The correct answer is: A 33-year-old woman delivered a female baby 4 months ago and is breast-feeding. She has now been diagnosed with bipolar disorder. You have decided to start her on a mood stabilizer. Which one should be avoided in this case? → Lithium, A 29-year-old mother who is currently nursing her 6-month-old child has now been diagnosed with depression. You are keen to start her on SSRIs for her

depression → Paroxetine, A 27-year-old breast-feeding mother has been diagnosed with depression. She has had depression in the past and responded to one of the older antidepressants. She insists trying one of the older drugs again. → Amitriptyline



Question 41

Not answered

Marked out of 5.00

Sleep disorders

Identify the correct type of sleep disorders, using the clinical descriptions provided below:

This is a disorder characterized by excessive daytime sleepiness, often disturbed night time sleep and disturbances in REM sleep

This disorder is characterized by sudden repeated contractions of one or more muscle groups during sleep, which is often followed by partial or complete arousal

This disorder is characterized by excessive somnolence, voracious appetite and hyper sexuality.

This disorder is characterized by a patient suddenly getting up screaming from sleep with features of autonomic arousal features e.g., tachycardia, sweating and anxiety. He or she may be difficult to arouse and rarely recall the episode on awakening.

This disorder is characterized by fearful dreams in the last one third of the night during REM sleep. The person wakes up very frightened and remembers the dream vividly.

Your answer is incorrect.

Explanation:

Narcolepsy: The cardinal symptoms of narcolepsy are excessive daytime drowsiness with sudden onset of REM sleep, often accompanied by cataplexy (sudden loss of muscle tone, provoked by strong emotion). Sleep paralysis and hypnagogic hallucinations may also occur (between wakefulness and sleep) but less commonly in the syndrome.

Kleine-Levin syndrome (KLS) is a rare disorder characterised by periodic episodes of hypersomnolence and hyperphagia. Associated features of the disorder include a lack of concentration, mood changes, sometimes hypersexuality and anxiety.

Nocturnal Myoclonus is characterized by sudden repeated contractions of one or more muscle groups during sleep, which is often followed by partial or complete arousal. The patient is usually not aware of myoclonus and often observed by the bed partner. The person usually complains of non-restorative sleep or of frequent awakenings. Due to night time insomnia, day time hypersomnia can occur and at times,

may be the only presenting symptom.

Sleep terrors or night terrors are characterized by patient suddenly getting up screaming from sleep with autonomic arousal features like tachycardia, sweating. He may be difficult to arouse and rarely recalls the episode on awakening. A night terror occurs in the early one third of the night and is a stage 4 n-REM disorder.

Nightmares are fearful dreams occurring in the last one third of the night during REM sleep. The person wakes up very frightened and remembers the dream vividly.

The correct answer is: This is a disorder characterized by excessive daytime sleepiness, often disturbed night time sleep and disturbances in REM sleep → Narcolepsy, This disorder is characterized by sudden repeated contractions of one or more muscle groups during sleep, which is often followed by partial or complete arousal → Nocturnal Myoclonus, This disorder is characterized by excessive somnolence, voracious appetite and hyper sexuality. → Kleine Levin syndrome, This disorder is characterized by a patient suddenly getting up screaming from sleep with features of autonomic arousal features e.g., tachycardia, sweating and anxiety. He or she may be difficult to arouse and rarely recall the episode on awakening. → Night terrors, This disorder is characterized by fearful dreams in the last one third of the night during REM sleep. The person wakes up very frightened and remembers the dream vividly. → Nightmares



Question 42

Not answered

Marked out of 5.00

Cardiovascular diseases & Psychotropics

Choose the most appropriate drug for each of the following situations:

A 75-year-old man has been diagnosed with late onset psychosis. He has a history of persistent atrial fibrillation

Choose...

A 65-year-old woman with atrial fibrillation presents with low mood, inability to sleep and poor concentration

Choose...

A 75-year-old man has presented with paranoia and auditory hallucinations. He has a history of cardiac arrhythmias

Choose...

A 63-year-old man with history of cardiac arrhythmias is now presenting with symptoms of clinical depression

Choose...

A 44-year-old man has suffered a myocardial infarction three weeks ago. Following this he became depressed and is currently suicidal

Choose...

Your answer is incorrect.

Explanation:

In the presence of persistent atrial fibrillation (AF), Aripiprazole may be an appropriate choice. AF is reported with clozapine, olanzapine and paliperidone.

This patient has symptoms of clinical depression. SSRIs may be suitable despite paroxysmal AF but beware of interaction with aspirin and warfarin. Avoid tricyclics in coronary disease. Tricyclics may also provoke AF.

In psychotic patients with arrhythmias, high doses of antipsychotics should be avoided. Risperidone may be least likely to cause conduction problems. (Ref: Oxford handbook of psychiatry- pg 882).

In depressed patients with arrhythmias, SSRIs should be first choice (but not citalopram or fluvoxamine).

In post MI depression, antidepressants should be best avoided in the first 2 months. If clinically indicated, SSRIs should be used rather than tricyclic antidepressants with the exception of citalopram and fluvoxamine. If sedation is required, consider use of a small dose of trazodone at night. (ref: Maudsley prescribing guidelines- pg 434)

The correct answer is: A 75-year-old man has been diagnosed with late onset psychosis. He has a history of persistent atrial fibrillation → Aripiprazole, A 65-year-old woman with atrial fibrillation presents with low mood, inability to sleep and poor concentration → SSRIs, A 75-year-old man has presented with paranoia and auditory hallucinations. He has a history of cardiac arrhythmias → Risperidone, A 63-year-old man with history of cardiac arrhythmias is now presenting with symptoms of clinical depression → SSRIs, A 44-year-old man has suffered a myocardial infarction three weeks ago. Following this he became depressed and is currently suicidal → SSRIs



Question 43

Not answered

Marked out of 4.00

Psychiatry & Endocrine disorders (2)

Using the clinical descriptions given below, identify the most probable endocrine disorder from list

A 30-year-old woman complains of vomiting, headaches, abdominal pain and reddish colouration of urine. She is labile in her mood and often becomes confused during these episodes. This usually happens whenever she has a night out with her friends

Choose...



A 21-year-old gentleman with polyuria, polydipsia, high plasma osmolality with low urine osmolality.

Choose...



A 36-year-old woman with hirsutism, weight gain and hypertension. She is also clinically depressed.

Choose...



A 37-year-old gentleman with palpitations, tremor, excessive sweating and weight loss. He also suffers from symptoms of anxiety.

Choose...



Your answer is incorrect.

Explanation:

In acute porphyria attacks almost always start with severe pain which is usually in the abdomen but may also be felt in the back or thighs. Nausea, vomiting and constipation are common. Some people may become very confused during an acute attack and behave in ways that are unusual for them. Drugs, alcohol or hormonal changes, for example, those associated with the menstrual cycle, often provoke acute attacks.

Diabetes insipidus is due to impaired water resorption by the kidney because of reduced ADH secretion from the posterior pituitary (cranial DI) or impaired response of the kidney to ADH (Nephrogenic DI). This condition is characterised by polyuria and polydipsia. Plasma osmolality should be high and urine osmolality being low.

Cushing's syndrome is characterised by weight gain, menstrual irregularities, amenorrhoea, hirsutism, depression, muscle weakness, fatigue and fractures.

Hyperthyroidism is characterised by weight loss despite increased appetite, irritability, heat intolerance, sweating, diarrhoea, tremor, frenetic activity, anxiety, emotional lability, psychosis, itch, oligomenorrhoea.



The correct answer is: A 30-year-old woman complains of vomiting, headaches, abdominal pain and reddish colouration of urine. She is labile in her mood and often becomes confused during these episodes. This usually happens whenever she has a night out with her friends → Acute porphyria, A 21-year-old gentleman with polyuria, polydipsia, high plasma osmolality with low urine osmolality. → Diabetes insipidus, A 36-year-old woman with hirsutism, weight gain and hypertension. She is also clinically depressed. → Cushing's syndrome, A 37-year-old gentleman with palpitations, tremor, excessive sweating and weight loss. He also suffers from symptoms of anxiety. → Hyperthyroidism



Question 44

Not answered

Marked out of 4.00

Liaison consultation psychiatry (1)

For each description below choose ONE option from the given list

A 40-year-old man presented with impulsivity and recent change of personality. His wife has mentioned that his father and grandfather also developed the same condition.

Choose... ▾

A 50-year-old farmer presents with low mood and irritability. On his chest, he has a rash of size 4 cm in diameter with clear margins.

Choose... ▾

A 50-year-old man develops nodules on the shin and also respiratory problems. He presents with depression and bilateral facial palsy.

Choose... ▾

A 50-year-old woman presents with abdominal pain, weight loss and jaundice. She also presents with signs of clinical depression with a sense of imminent doom.

Choose... ▾

Your answer is incorrect.

Explanation:

Huntington's disease is an autosomal dominant disorder with high penetrance. It is a progressive neurodegenerative disorder with chorea and dystonia, incoordination, cognitive decline, personality change and behavioural difficulties. The onset of symptoms is often in middle age after affected individuals have had children.

The diagnosis is Lyme disease, which is caused by *Borrelia burgdorferi*, transmitted via deer tick bite. A red spot develops initially around the location (crotch or armpit). This will develop a central clearing called erythema migrans within 4 weeks. Patients with late-stage Lyme disease may present with a variety of neurological and psychiatric problems, ranging from mild to severe. These include Cognitive deficits, Psychosis, Seizures, Violent behavior, irritability, Anxiety and Depression. This is rare in UK; more common in North America.

Neurosarcoidosis is characterized by idiopathic granulomas in various tissues - mainly lungs and mediastinal node causing respiratory problems. It may affect CNS especially cranial nerves producing bilateral facial palsy. Depression is seen in 20%. Rarely psychosis can occur. Patients may have erythema nodosum on their shin - a cutaneous sign.

Pancreatic carcinoma is more often seen in males than females. The patients present with abdominal pain, weight loss, weakness and jaundice. Depression with a sense of imminent doom but without severe guilt is a cardinal feature. There will be also loss of drive and motivation.

The correct answer is: A 40-year-old man presented with impulsivity and recent change of personality. His wife has mentioned that his father and grandfather also developed the same condition. → Huntington's disease, A 50-year-old farmer presents with low mood and irritability. On his chest, he has a rash of size 4 cm in diameter with clear margins. → Lyme's disease, A 50-year-old man develops nodules on the shin and also respiratory problems. He presents with depression and bilateral facial palsy. → Neurosarcoidosis, A 50-year-old woman presents with abdominal pain, weight loss and jaundice. She also presents with signs of clinical depression with a sense of imminent doom. → Cancer pancreas



Question 45

Not answered

Marked out of 3.00

Liaison consultation psychiatry (2)

For each description below choose ONE correct option from the given list

A 44-year-old homeless man is brought into A&E. He is agitated, psychotic and depressed. The EEG shows diffusely decreased alpha activity and his gait is disturbed. MRI shows hypointensity of the striatum.

Choose...

A 25-year-old man recently returned from America shows aggressive behaviour. He recently developed seizures and has periods of decreased consciousness. His full blood count, lumbar puncture and liver functions are normal

Choose...

A 30-year-old woman has bilateral basal ganglia calcification. She is being investigated for renal stones and polyuria.

Choose...

Your answer is incorrect.

Explanation:

Fahr's disease is due to an idiopathic (autosomal dominant) progressive calcium deposition in the basal ganglia. An onset between the ages of 20 and 40 is associated with schizophreniform psychoses and catatonic symptoms. An onset between the ages of 40 and 60 is associated with dementia and choreoathetosis. Depression is also very common, but mania is uncommon. 50% patients report psychiatric problems. Psychiatric symptoms correlate with more extensive calcification. The pattern of cognitive impairment found in FD is of the frontosubcortical type. The commonest neurological features of FD are parkinsonism, chorea, dystonia, tremor, gait disturbance, dysarthria, seizures and myoclonus. MRI shows hypointensity of striatum. EEG is non specific - may show diffuse changes. Fahr's disease is also called familial cerebral ferrocalsinosis. NOTE: Though sometimes used interchangeably, it is important to differentiate the specific presentation of Fahr's disease from the more generic description of Fahr' syndrome that refers to bilateral striato-pallido-dentate calcifications (see below).

In complex partial seizures, investigations are normal. Decreased periods of consciousness is common. Complex partial seizures begin as simple partial seizures and progress to impairment of consciousness. There may be also impairment of consciousness at the onset. Aggressive behaviour is also a possibility in patients with complex partial seizures.

Fahr's syndrome is due to specific causes of calcium deposition in the basal ganglia such as hypoparathyroidism featured by renal stones and polyuria. Radiological basal ganglia calcification without clinical features on CT may occur at a rate of about 0.9%.

The correct answer is: A 44-year-old homeless man is brought into A&E. He is agitated, psychotic and depressed. The EEG shows diffusely decreased alpha activity and his gait is disturbed. MRI shows hypointensity of the striatum. → Fahr's syndrome, A 25-year-old man recently returned from America shows aggressive behaviour. He recently developed seizures and has periods of decreased consciousness. His full blood count, lumbar puncture and liver functions are normal → Complex partial seizures, A 30-year-old woman has bilateral basal ganglia calcification. She is being investigated for renal stones and polyuria. → Fahr's syndrome



Question 46

Not answered

Marked out of 4.00

Infectious Diseases & Psychiatry

For each description below choose ONE most appropriate option from the given list

A 55-year-old gentleman has developed psychotic depression. He has paraesthesia involving both legs and has high stepping gait and pupillary abnormalities

Choose... ▾

A man who frequently flies to the United States reports intense fatigue and musculoskeletal pain. Initially he was diagnosed to have fibromyalgia. He now reports a rash somewhere in his trunk with a central reddish spot. This does not respond to topical preparations offered by his GP.

Choose... ▾

A 50 year old gentleman with fever, amnesia, aphasia, weakness, wandering and admitted to A&E with vivid and colourful hallucinations

Choose... ▾

A 36 year old gentleman who is a intravenous drug user now presenting with mental slowness, loss of concentration and fatuous mood

Choose... ▾

Your answer is incorrect.

Explanation:

Case 1 is suggestive of syphilis caused by *Treponema pallidum*. High stepping gait and pupil abnormalities are features of complicated syphilitic infection.

Case 2 refer to Lyme disease. It is caused by *Borrelia burgdorferi*, transmitted via deer tick bite. A red spot develops initially around the location (crotch or armpit). This will develop a central clearing called erythema migrans within 4 weeks. About 15% develop neuroborreliosis, where CNS is affected. Back pain, which is worse at night, facial numbness and facial palsy, may develop. Patients with late-stage Lyme disease may present with a variety of neurological and psychiatric problems, ranging from mild to severe.

Case 3 is an example of Herpes simplex encephalitis. This is the commonest identified cause of viral encephalitis and the commonest cause of limbic encephalitis - affects temporal lobe and limbic circuit. 70% of cases are caused by herpes simplex type 1. 70% show psychiatric disturbances including acute confusion, depression and psychosis.

Case 4 suggests HIV encephalopathy. Early cognitive features include forgetfulness, loss of concentration, mental slowing, and reduced performance on sequential mental activities. Other features include apathy, reduced spontaneity and emotional responsivity, and social withdrawal. Depression, irritability or emotional lability, agitation, and psychotic symptoms may also occur.

The correct answer is: A 55-year-old gentleman has developed psychotic depression. He has paraesthesia involving both legs and has high stepping gait and pupillary abnormalities → Treponema pallidum, A man who frequently flies to the United States reports intense fatigue and musculoskeletal pain. Initially he was diagnosed to have fibromyalgia. He now reports a rash somewhere in his trunk with a central reddish spot. This does not respond to topical preparations offered by his GP. → Borrelliosis, A 50 year old gentleman with fever, amnesia, aphasia, weakness, wandering and admitted to A&E with vivid and colourful hallucinations → Herpes simplex, A 36 year old gentleman who is a intravenous drug user now presenting with mental slowness, loss of concentration and fatuous mood → HIV



Question 47

Not answered

Marked out of 4.00

Metabolic disturbances (2)

Choose one option for each of the following situations

A 46-year-old gentleman with metabolic syndrome was started on Olanzapine 4 weeks ago. He is confused, agitated and has abdominal pain

Choose...

A 23-year-old woman has a history of eating disorder. She complains of lethargy, palpitations and weakness in her arms and legs

Choose...

A 46-year-old woman was started on fluoxetine 4 weeks ago. The dosage was increased from 20 to 40 mg last week. She complains of nausea, dizziness and lethargy

Choose...

Following thyroid surgery, a 45-year-old woman became depressed and irritable. She was observed to have twitchy movements in her face

Choose...

Your answer is incorrect.

Explanation:

Q1: Olanzapine can cause insulin resistance and induce hyperglycemia. This man presents with features of ketoacidosis, which is characterized by agitation, confusion and abdominal pain. In patients with eating disorder, vomiting and purging could cause hypokalemia.

Q2: The signs of hypokalemia include muscle weakness, hypotonia, cardiac arrhythmias, cramps and tetany due to alkalosis.

Q3: This is hyponatraemia following treatment with antidepressants like fluoxetine. The signs of hyponatraemia include dizziness, nausea, lethargy, confusion, cramps and seizures.

Q4: This patient has developed features of hypoparathyroidism following thyroid surgery. The common features include depression, anxiety, irritability, acute organic reaction and cognitive impairment. The psychiatric symptoms increase as hypocalcemia worsens but they respond rapidly to treatment with vitamin D. The physical symptoms include tetany, perioral anesthesia, carpopedal spasm and tapping over parotid causes facial muscles to twitch (Chvostek's sign).

The correct answer is: A 46-year-old gentleman with metabolic syndrome was started on Olanzapine 4 weeks ago. He is confused, agitated and has abdominal pain → Hyperglycemia, A 23-year-old woman has a history of eating disorder. She complains of lethargy, palpitations and weakness in her arms and legs → Hypokalemia, A 46-year-old woman was started on fluoxetine 4 weeks ago. The dosage was increased from 20 to 40 mg last week. She complains of nausea, dizziness and lethargy → Hyponatraemia, Following thyroid surgery, a 45-year-old woman became depressed and irritable. She was observed to have twitchy movements in her face → Hypocalcaemia



Question 48

Not answered

Marked out of 5.00

Disorders in Liaison Psychiatry (2)

Using the simple definitions given below, identify the type of disorder being described

Physical effects of a conflict

Choose... ▾

Mental effects of a conflict

Choose... ▾

Presents with physical symptoms with no organic basis, but the patient resists discussing psychological problems

Choose... ▾

Intentional production of symptoms often for the monetary benefits associated with being ill.

Choose... ▾

Presents with physical symptoms with no organic basis but there is no monetary gain; the only gain is assumption of a sick role.

Choose... ▾

Your answer is incorrect.

Explanation:

Dissociation refers to the mental effects of a conflict. It can present as amnesia, loss of identity, or a parallel but altered personality.

Conversion refers to the physical effects of a conflict that presents as loss of a previously normal neurological function e.g., a paralysis, blindness, ataxia, anaesthesia, aphonia or seizures.

Somatoform disorder refers to the production of a focal symptom while **somatisation disorder** is usually polysymptomatic e.g., pain, vomiting etc.

Malingering is the intentional feigning of symptoms, often for monetary benefits such as compensation claims etc. Factitious disorder is a truly puzzling condition. The only gain appears to be the assumption of a sick role; it is often seen in professionals with some medical knowledge.

Munchausen syndrome is the severe form of factitious disorder that is associated with a wide doctor-shopping.

The correct answer is: Physical effects of a conflict → Conversion disorder, Mental effects of a conflict → Dissociation disorder, Presents with physical symptoms with no organic basis, but the patient resists discussing psychological problems → Somatisation disorder, Intentional production of symptoms often for the monetary benefits associated with being ill. → Malingering, Presents with physical symptoms with no organic basis but there is no monetary gain; the only gain is assumption of a sick role. → Factitious disorder



Question 49

Not answered

Marked out of 4.00

Psychiatric syndromes associated with stroke

For each of the following clinical conditions below, match the associated psychiatric syndrome from the list;

A 78-year-old gentleman with lacunar infarcts presenting with apraxia, reduced attention and visuospatial neglect

Choose... ▾

83-year-old lady with history of recurrent transient ischaemic attacks presenting with bursts of aggressive behaviour, anxiety and episodes of crying when asked to attempt solving crosswords

Choose... ▾

80-year-old gentleman with right-sided hemiparesis presents with impairment in the control of crying

Choose... ▾

75-year-old gentleman with thromboembolic strokes involving basal ganglia presents with anhedonia, avolition and lack of motivation but has no tearfulness or pessimism.

Choose... ▾

Your answer is incorrect.

Explanation:

Apraxia, reduced attention and visuo-spatial neglect are suggestive of stroke related **cognitive** impairment.

Catastrophic reaction is a response often seen in patients with dementia who are asked to perform tasks beyond their, now impaired, performance level. There is sudden agitation, anger and occasionally violence.

An increase in the frequency of crying (shedding tears, sobbing) or laughing episodes that are often uncontrollable is called **emotionalism**. Post stroke emotionalism has a high prevalence rate varying between 15% to 35%.

Apathy refers to the presence of dull emotional tone associated with detachment, which mimics depression. Often there is avolition and poor motivation. No other overt symptoms of depression are present. This is seen in 20% of patients with stroke.

The correct answer is: A 78-year-old gentleman with lacunar infarcts presenting with apraxia, reduced attention and visuospatial neglect → Cognitive impairment, 83-year-old lady with history of recurrent transient ischaemic attacks presenting with bursts of aggressive behaviour, anxiety and episodes of crying when asked to attempt solving crosswords → Catastrophic reaction, 80-year-old gentleman with right-sided hemiparesis presents with impairment in the control of crying → Emotionalism, 75-year-old gentleman with thromboembolic strokes

involving basal ganglia presents with anhedonia, avolition and lack of motivation but has no tearfulness or pessimism. → Apathy without depression



Question 50

Not answered

Marked out of 4.00

Neuropsychiatric complications of Parkinson's disease

For each of the following Parkinsonian conditions below, match the most commonly associated neuropsychiatric complication.

An 84-year-old gentleman with poor education, with late onset Parkinson's disease exhibits severe extra pyramidal signs

Choose...

A 76-year-old gentleman with late-onset Parkinson's disease on high dose levodopa and selegiline still exhibits several extrapyramidal signs

Choose...

A 73-year-old gentleman with a previous history of bipolar affective disorder is on levodopa and pramiprexole

Choose...

A 63-year-old woman with prominent right-sided extrapyramidal signs, notable bradykinesia and gait disturbances. She is significantly impaired in activities of daily living.

Choose...

Your answer is incorrect.

Explanation:

The risk factors for **cognitive impairment** in patients with Parkinson's disease include older patients, late-onset parkinson's disease, poor educational achievements and severe extrapyramidal signs.

Psychosis in Parkinson's disease is mostly drug related. Dopaminergic drugs such as levodopa and Selegiline can cause psychotic symptoms such as paranoia and hallucinations-especially visual modality.

Pramiprexole can cause exacerbation of mood symptoms and could precipitate **hypomania**/mania. Denovo mania is rare in Parkinson's. It is mostly associated with levodopa and dopamine agonist treatment especially in pre-existing bipolar affective disorder/ family history of bipolar disorder. Levodopa can result in pathological gambling, hypersexuality and hallucinations in addition to hypomania.

The risk factors for developing **depression** in parkinson's disease include female sex, younger age of onset, prominent right-sided signs, bradykinesia and gait disturbance, rapid disease progression, poorer cognitive status and activities of daily living

The correct answer is: An 84-year-old gentleman with poor education, with late onset Parkinson's disease exhibits severe extra pyramidal signs → Cognitive impairment, A 76-year-old gentleman with late-onset Parkinson's disease on high dose levodopa and selegeline still exhibits several extrapyramidal signs → Psychosis, A 73-year-old gentleman with a previous history of bipolar affective disorder is on levodopa and pramiprexole → Hypomania, A 63-year-old woman with prominent right-sided extrapyramidal signs, notable bradykinesia and gait disturbances. She is significantly impaired in activities of daily living. → Depression



Question 51

Not answered

Marked out of 5.00

Movement disorders in psychiatry

For each of the following clinical conditions below, match the most appropriate diagnosis from the given list.

A 65-year-old woman presents with short strides, short stepping gait and progressively small and cramped handwriting when tested. She has a vertical gaze problem on examination.

Choose...

A 15-year-old boy develops non-repetitive sudden movements of his arms. He has no past history of similar problems, and the family has no degenerative illnesses. The only medical history seems to be an episode of pharyngitis nearly 6 months ago.

Choose...

A 45-year-old man has a history of progressive memory loss. His wife has observed him having sudden muscle movements on a number of occasions as if experiencing an electric shock momentarily.

Choose...

A 37-year-old woman has repeated involuntary blinking and chin-thrusting. She is cognitively intact and has no other movement problems. She constantly uses chewing gums to conceal her facial movements.

Choose...

A 57-year-old gentleman presents with long strides and quick involuntary knee flexion. He has history of seizures and cognitive impairment.

Choose...

Your answer is incorrect.

Explanation:

Q1: Patients with Parkinson's disease usually have short strides, short stepping gait and progressively small and cramped handwriting.

Q2: Sydenham's chorea usually occurs following beta hemolytic streptococcal infection (pharyngitis). It is a disease characterized by rapid, uncoordinated jerking movements affecting primarily the face, feet and hands.

Q3: The diagnosis is Crutzfeldt Jakob disease. This disease is characterized by myoclonus, which are brief asynchronous muscular jerks of the extremities.

Q4: Meige's syndrome is oral facial dystonia characterized by repetitive blinking and chin thrusting. Some patients have lip pursing or tongue movements and, for a few, the movements spread into the shoulders.

Q5: Neuroacanthocytosis is characterized by a peculiar gait of lurching with long strides and quick, involuntary knee flexion. Seizures, generally tonic-clonic are noted. A subcortical dementia may set in. It is a genetically heterogenous neurologic disorder characterized with acanthocytosis. Neurologic problems usually consist of movement disorders or ataxia, personality changes, cognitive deterioration, axonal neuropathy, and seizures.

The correct answer is: A 65-year-old woman presents with short strides, short stepping gait and progressively small and cramped handwriting when tested. She has a vertical gaze problem on examination. → Parkinson's disease, A 15-year-old boy develops non-repetitive sudden movements of his arms. He has no past history of similar problems, and the family has no degenerative illnesses. The only medical history seems to be an episode of pharyngitis nearly 6 months ago. → Sydenham's chorea, A 45-year-old man has a history of progressive memory loss. His wife has observed him having sudden muscle movements on a number of occasions as if experiencing an electric shock momentarily. → Crutzfeldt Jakob disease, A 37-year-old woman has repeated involuntary blinking and chin-thrusting. She is cognitively intact and has no other movement problems. She constantly uses chewing gums to conceal her facial movements. → Meige's syndrome, A 57-year-old gentleman presents with long strides and quick involuntary knee flexion. He has history of seizures and cognitive impairment. → Neuroacanthocytosis



Question 52

Not answered

Marked out of 4.00

Psychosomatic disorders

Identify the most appropriate diagnosis for each of the following clinical situations

A 30-year-old woman presents with hyperpigmented buccal mucosa. She has also lost weight of late, which she considers on a positive note. She reports some nausea in the mornings, abdominal pains, feeling dizzy on and off and feeling confused at times.

Choose...



A 55-year-old woman presents with weight loss, foul smelling bulky stools, abdominal pain, bloating, mouth ulcers and vomiting. Pathology shows villous atrophy on jejunal biopsy.

Choose...



A 24-year-old lady has weight loss and irregular periods. She also has tremors and frequent stools of normal consistency. She wants to turn off the heating even in the coldest of days.

Choose...



A 27-year-old woman has weight gain and hirsutism. She is also clinically depressed and has a history of hypertension.

Choose...



Your answer is incorrect.

Explanation:

Addison's disease: Clinical features include weakness, abdominal pain, anorexia, weight loss, constipation, myalgia and depression. Signs include hyperpigmentation (palmar creases, buccal mucosa), vitiligo and postural hypotension.

Coeliac disease: Clinical presentations include abdominal pain, steatorrhoea, bloating, nausea/vomiting, aphthous ulcers, angular stomatitis, weight loss and fatigue. Jejunal biopsy shows villous atrophy, reversing on gluten free diet.

Hyperthyroidism: Clinical symptoms include weight loss despite increased appetite, heat intolerance, sweating, diarrhoea, tremor, irritability, emotional lability, psychosis, oligomenorrhoea, itch. Signs include atrial fibrillation, warm peripheries, fine tremor, goitre, palmar erythema, hair thinning and lid-lag.

Cushing's syndrome: Clinical features include weight gain, menstrual irregularity, amenorrhoea, hirsutism in females, impotence in males, depression, muscle weakness and fractures. Signs include tissue wasting, myopathy, thin skin, purple abdominal striae, osteoporosis, hirsutism, hypertension, hyperglycemia, predisposition to infection and poor wound healing.

The correct answer is: A 30-year-old woman presents with hyperpigmented buccal mucosa. She has also lost weight of late, which she considers on a positive note. She reports some nausea in the mornings, abdominal pains, feeling dizzy on and off and feeling confused at times. → Addison's disease, A 55-year-old woman presents with weight loss, foul smelling bulky stools, abdominal pain, bloating, mouth ulcers and vomiting. Pathology shows villous atrophy on jejunal biopsy. → Celiac disease, A 24-year-old lady has weight loss and irregular periods. She also has tremors and frequent stools of normal consistency. She wants to turn off the heating even in the coldest of days. → Hyperthyroidism, A 27-year-old woman has weight gain and hirsutism. She is also clinically depressed and has a history of hypertension. → Cushing's disease



Question 53

Not answered

Marked out of 5.00

Investigations

Choose the most appropriate investigations of choice for the following clinical situations.

A 35-year-old gentleman presenting with fever, abrupt onset confusion and behavioral changes

Choose... ▾

A 75-year-old gentleman, who suffers from late onset psychosis, has been on risperidone. He develops parkinsonian symptoms and neurologist says that it could be drug-induced

Choose... ▾

A 46-year-old gentleman comes to your outpatients' clinic, as he is transferred from another area. He has a history of schizoaffective disorder. He is on quetiapine 700 mg/day and amitriptyline 75 mg/day

Choose... ▾

A 39-year-old woman has a history of bipolar affective disorder and is on sodium valproate 1000mg/day. She has developed nausea and abdominal pain radiating to the back.

Choose... ▾

A 55-year-old man with chronic schizophrenia is on 750 mg of clozapine. His brother has noticed bite marks in his tongue and episodes of confusion.

Choose... ▾

Your answer is incorrect.

Explanation:

Q1: The diagnosis could be Herpes simplex encephalitis which presents with fairly abrupt onset of confusion, memory impairment, and often seizures. 70% show psychiatric disturbances including acute confusion, depression and psychosis. Fever is common but not invariable. Magnetic resonance imaging (MRI) provides the most sensitive method of detecting early lesions and is the imaging of choice in HSE.

Q2: This could be a case of Lewy body dementia presenting initially with psychotic symptoms and later developed other features like motor symptoms of parkinsonism and neuroleptic sensitivity. Alternatively the patient might be now developing parkinson's disease on top of functional psychotic illness. In either cases the investigation useful would be DAT-SCAN (I-123-FP-CITscan). This uses SPECT technology and can be undertaken in most imaging departments. This demonstrated abnormal images in 86% of DLB cases and also helps to differentiate idiopathic Parkinson's from drug-induced parkinsonism.

Q3: Many psychotropic drugs such as quetiapine and tricyclic antidepressants like amitriptyline are associated with ECG changes (QTc prolongation) and are causally linked to ventricular arrhythmias and sudden cardiac death.

Q4: Sodium valproate can cause both gastric irritation and hyperammonemia, both of which can lead to nausea. It can also cause pancreatitis, during which the patient can present with abdominal pain radiating to the back. Investigation of choice is CT abdomen.

Q5: Clozapine on high doses (more than 600 mg/day) can induce seizures. Patients taking clozapine may have generalized seizures with loss of consciousness, tongue biting and incontinence. EEG would be helpful to look for any epileptiform activity.

The correct answer is: A 35-year-old gentleman presenting with fever, abrupt onset confusion and behavioral changes → MRI brain, A 75-year-old gentleman, who suffers from late onset psychosis, has been on risperidone. He develops parkinsonian symptoms and neurologist says that it could be drug-induced → SPECT scan, A 46-year-old gentleman comes to your outpatients' clinic, as he is transferred from another area. He has a history of schizoaffective disorder. He is on quetiapine 700 mg/day and amitriptyline 75 mg/day → ECG, A 39-year-old woman has a history of bipolar affective disorder and is on sodium valproate 1000mg/day. She has developed nausea and abdominal pain radiating to the back. → CT scan abdomen, A 55-year-old man with chronic schizophrenia is on 750 mg of clozapine. His brother has noticed bite marks in his tongue and episodes of confusion. → EEG



Question 54

Not answered

Marked out of 5.00

Depression & Neuroliaison psychiatry

Choose the most appropriate first line agents for each of the following clinical situations:

Depression in post stroke patients	Choose... 
Depression in multiple sclerosis	Choose... 
Depression in Parkinson's disease	Choose... 
Depression in HIV	Choose... 
Depression in epilepsy	Choose... 

Your answer is incorrect.

Explanation:

The prevalence of post stroke depression is around 35%. Consider a trial of antidepressant medication in those with persistently depressed mood of at least 1 months duration. Both fluoxetine and citalopram have good RCT evidence to support their use.

The lifetime prevalence of depression in multiple sclerosis is 50%. SSRIs should be first line treatment and sertraline is shown to be especially effective.

In Parkinson's disease, approximately 25% will suffer from major depression at some point during the course of their illness and a further 25% from milder forms of depression. SSRIs are considered to be first line treatment. Depression in Parkinson's disease predicts greater cognitive decline, deterioration in cognitive declining and progression of motor symptoms.

Depression is common in individuals with HIV and first line agents include SSRIs such as citalopram.

Depressive symptoms occur in 30-50% of patients with epilepsy. SSRIs are the first line, but may reduce seizure threshold in some. Moclobemide is also considered as a good choice.

The correct answer is: Depression in post stroke patients → Selective serotonin reversible inhibitor, Depression in multiple sclerosis → Selective serotonin reversible inhibitor, Depression in Parkinson's disease → Selective serotonin reversible inhibitor, Depression in HIV → Selective serotonin reversible inhibitor, Depression in epilepsy → Selective serotonin reversible inhibitor



Question 55

Not answered

Marked out of 5.00

Suicide statistics (1)

Choose one most appropriate option from the list to indicate the correct data on suicides

The percentage of preventable deaths according to mental health teams in England

Choose... ▾

Risk of suicide within one year of deliberate self harm

Choose... ▾

The percentage of older patients who committed suicide within 1 week of visiting their primary care physician

Choose... ▾

The percentage of suicides that occurred within 3 months of discharge from an inpatient unit

Choose... ▾

The percentage of patients who committed suicide and who visited their GP in last one week

Choose... ▾

Your answer is incorrect.

Explanation:

The percentage of preventable deaths according to mental health teams in England is 22% (especially inpatient suicides).

The risk of suicide within one year of deliberate self harm is 0.7% (nearly 1 in 100); more in males-1.1%, 0.5% in females.

The percentage of older patients who committed suicide within 1 week of visiting their primary care physician is 20% on the same day as their suicide, 40% within 1 week and 70% within 1 month.

The percentage of suicides that occurred within 3 months of discharge from an inpatient unit is 25%. Among them, 40% of these occurred before first follow up.

The percentage of patients who committed suicide, who visited their GP in last one week is 40%, having seen by GP in last 4 weeks is 66%.

The correct answer is: The percentage of preventable deaths according to mental health teams in England → 22%, Risk of suicide within one year of deliberate self harm → 0.7%, The percentage of older patients who committed suicide within 1 week of visiting their primary care

physician → 40%, The percentage of suicides that occurred within 3 months of discharge from an inpatient unit → 25%, The percentage of patients who committed suicide and who visited their GP in last one week → 40%



Question 56

Not answered

Marked out of 4.00

Investigations in schizophrenia

Choose the best investigative approach to study various epidemiological aspects of schizophrenia as described below:

Separating environmental factors from genetic factors

Choose...



The presence of neurological or motor abnormalities that predate onset of illness

Choose...



Identification of cognitive and IQ deficits before the illness onset

Choose...



Delineating the natural course of the illness

Choose...



Your answer is incorrect.

Explanation:

The observation that schizophrenia is more commonly observed among the relatives of individuals with schizophrenia than in the general population could be attributed to both genetic and shared environmental causes. Adoption designs permit evaluation of the role of genetic factors in schizophrenia independently of the influence of family environments.

Walker and Levine (1998) successfully employed the analysis of childhood home movies by blind raters to identify children who later developed schizophrenia on the basis of neuromotor abnormalities. This study suggested that preschizophrenic subjects can be distinguished from sibling control subjects within the first 8 years of life by observing their behavior. Am J Psychiatry 1990;147:1052-1056.

To examine associations between premorbid cognitive functioning in childhood or adolescence and subsequent schizophrenia, three broad categories of study have been employed: birth cohort designs with cognitive testing during childhood, army conscript designs with cognitive performance measured at conscription, and studies using school grades. (McCabe, Epidemiol Rev (2008) 30 (1): 77-83.)

To study the natural course of an illness, without any alterations induced by pharmacological or treatment agents, unmedicated cohorts must be observed prospectively.

The correct answer is: Separating environmental factors from genetic factors → Adoption design, The presence of neurological or motor abnormalities that predate onset of illness → Home movies, Identification of cognitive and IQ deficits before the illness onset → Study of

army conscripts, Delineating the natural course of the illness → Unmedicated cohorts



Question 57

Not answered

Marked out of 5.00

Liaison psychiatry (3)

You have been asked to see the following patients in your liaison psychiatry clinic. Select the one most likely diagnosis for each of the following clinical situations;

A 23-year-old woman is referred by her GP. She states that her eyes are set wide apart and people call her 'fish-face'. She spends hours in front of the mirror examining her eyes. She has seen surgeons twice to get plastic surgery done to her eyes. However there is no obvious deformity

Choose... ▾

A 21-year-old woman presents with excessive sweating, anxiety attacks and blushing when eating in public places and talking to others when attending parties. She is isolating herself now. She is extremely worried about her sister's marriage in a month's time, when she will have to meet lots of people.

Choose... ▾

A mother brought her son, a 23-year-old engineering student, to the outpatient clinic. He is complaining of sudden loss of vision and double vision on leftward gaze for last week or so. Eye examination and investigations are normal. His father died in a car accident 1 week ago, whilst he was driving the vehicle

Choose... ▾

A 36-year-old woman complains of severe tiredness for over 6 months and is unable to work. Rest does not alleviate it. There is no identifiable organic pathology.

Choose... ▾

A 29-year-old woman was referred by her GP as she thinks that she suffers from severe chronic pain in her bottom for last 6 months. Examination and investigations are normal. This is an imaginary pain in the bottom exactly in the sitting area. The pain actually started 4 months ago, when you went to the fertility clinic, as you are desperate to have a child. You have not been successful so far and obviously you are under a lot of stress.

Choose... ▾

Your answer is incorrect.

Explanation:

In body dysmorphic disorder there is an excessive concern (overvalued idea) about trivial or non-existent physical abnormalities, which are perceived to be deformities.

Social phobia consists of a fear of social interaction in a crowd and fear of appearing ridiculous when observed by others. Patients tend to avoid any social interaction and may even seek to avoid the gaze of others. It is associated with severe anticipatory anxiety. Drug or alcohol misuse is common.

In conversion disorder there is an alteration or loss of physical functioning suggestive of a physical disorder but which is an expression of psychological conflict or need with primary and secondary gain.

Chronic fatigue syndrome is characterized by persistent or relapsing unexplained chronic fatigue for at least 6 months, is of new or definite onset, is not the result of an organic disease or of continuing exertion and is not alleviated by rest. Fatigue results in a substantial reduction in previous occupational, educational, social, and personal activities. Four or more of the following symptoms, concurrently present for more than 6 months: impaired memory or concentration, sore throat, tender cervical or axillary lymph nodes, muscle pain, pain in several joints, new headaches, unrefreshing sleep, or malaise after exertion.

Somatoform pain disorder is characterized by severe persistent and distressing pain not fully explained by a physical disorder. Co-morbid psychiatric disorder such as depression is common. Often psychological factors (emotional factors) and psychosocial stressors play a role in precipitating and perpetuating the pain. It can lead to insomnia, inactivity and depression.

The correct answer is: A 23-year-old woman is referred by her GP. She states that her eyes are set wide apart and people call her 'fish-face'. She spends hours in front of the mirror examining her eyes. She has seen surgeons twice to get plastic surgery done to her eyes. However there is no obvious deformity → Body dysmorphic disorder, A 21-year-old woman presents with excessive sweating, anxiety attacks and blushing when eating in public places and talking to others when attending parties. She is isolating herself now. She is extremely worried about her sister's marriage in a month's time, when she will have to meet lots of people. → Social phobia, A mother brought her son, a 23-year-old engineering student, to the outpatient clinic. He is complaining of sudden loss of vision and double vision on leftward gaze for last week or so. Eye examination and investigations are normal. His father died in a car accident 1 week ago, whilst he was driving the vehicle → Conversion disorder, A 36-year-old woman complains of severe tiredness for over 6 months and is unable to work. Rest does not alleviate it. There is no identifiable organic pathology. → Chronic fatigue syndrome, A 29-year-old woman was referred by her GP as she thinks that she suffers from severe chronic pain in her bottom for last 6 months. Examination and investigations are normal. This is an imaginary pain in the bottom exactly in the sitting area. The pain actually started 4 months ago, when you went to the fertility clinic, as you are desperate to have a child. You have not been successful so far and obviously you are under a lot of stress. → Somatoform pain disorder

Question 58

Not answered

Marked out of 4.00

Epidemiological terms II

Identify the most appropriate epidemiological term that refers to each of the following commonly used descriptions

Risk of an illness	Choose... 
Burden of disease	Choose... 
Denominator of incidence density	Choose... 
Denominator of prevalence	Choose... 

Your answer is incorrect.

Explanation: In epidemiological terms, **incidence** is defined as the number of new cases of a disease in a population, divided by the total number of people at risk in the population per unit of time. Often when we refer to risk of a condition, we are talking about the incidence. This is also known as **cumulative incidence** or incidence rate. As not all those who are at risk can be followed up for long periods of time (due to attrition rate), a measure called **incidence density** which accounts for the varying time periods of follow up is often used in research. Although the numerator is the same as in the calculation of cumulative incidence, the denominator is now the sum of each individual's time at risk (**Person-Years**). In epidemiological terms, **prevalence** is defined as the number of existing cases of a disease in a population divided by total size of the population. Often when we refer to burden of a condition, we are talking about the prevalence.

The correct answer is: Risk of an illness → Incidence, Burden of disease → Prevalence, Denominator of incidence density → Person-Years, Denominator of prevalence → Mid year population

Question 59

Not answered

Marked out of 4.00

Epidemiology of self-harm

As a Year 1 Core Trainee, you are attending patients that present with episodes of self harm in your local A&E. Using the most widely accepted epidemiological data in this regard, identify the probabilities associated with each of the following scenario

The proportion of all episodes of self-harm that occur in people over the age of 65

Choose... ▾

The subsequent risk of suicide 10 or more years after an episode of self harm.

Choose... ▾

The proportion of individuals who will harm themselves again in their lifetimes

Choose... ▾

Proportion of individuals who self harm that will repeat at least 5 times in a given year

Choose... ▾

Your answer is incorrect.

Explanation: Following an act of self-harm the rate of suicide increases to between 50 and 100 times the rate of suicide in the general population (Hawton et al., 2003b; Owens et al., 2002). In other words, the subsequent risk of suicide is at least 3% after 10 or more years. The risk of repetition of self harm is extremely high; up to 40% will go on to repeat. About one in six people who attend an emergency department following self-harm will self-harm again in the following year (Owens et al., 2002); a small minority of people will do so repeatedly - 10% will repeat at least 5 times in a given year. This high-repetition group is one reason for the overall high repetition rate observed (40% repetition in 10 years or more). Self-harm can occur at any age but is most common in adolescence and young adulthood (Meltzer et al., 2002a). For example, based on a survey of 12,529 children and young people aged 5 years to 15 years, 1.3% had tried to harm themselves (data collected from parents) (Meltzer, 2002a). In contrast, only about 5% of all episodes of self-harm occur in people over the age of 65 (Dennis et al., 1997; Draper, 1996; Owens et al., 1991).

The correct answer is: The proportion of all episodes of self-harm that occur in people over the age of 65 → 5%, The subsequent risk of suicide 10 or more years after an episode of self harm. → 3%, The proportion of individuals who will harm themselves again in their lifetimes → 40%, Proportion of individuals who self harm that will repeat at least 5 times in a given year → 10%

Question 60

Not answered

Marked out of 3.00

Prevalence

Choose the lifetime prevalence for the following phenomena according to Office of National Statistics UK Household Survey in 2000 (aged 16 to 74).

DSH without suicidal intent

Choose... ▾

Suicide attempts

Choose... ▾

Suicidal thoughts

Choose... ▾

Your answer is incorrect.

Explanation: In this survey, overall 14.9% of survey respondents said they had considered suicide at some point in their life, 3.9% in the past year and 0.4% had done so in the past week. White respondents, both men and women, were about twice as likely to have had suicidal thoughts compared with the sample of Black or South Asian origin: 15% compared with 8%. But there was no statistically significant variation in the percentage of respondents in different ethnic groups who had ever attempted suicide. 28% of divorced women and 25% of divorced men said they had ever had suicidal thoughts compared with 13% of married women and 9% of married men. All respondents were asked if they had ever deliberately harmed themselves without suicidal intent, and 2% said that they had (2% of men and 3% of women). 4.4% of respondents said they had ever attempted suicide (both with and without intent), and 0.5% had attempted suicide in the past year. The two factors which seemed to have the greatest influence on suicide attempts were number of stressful life events and psychosis. Singleton N, Bumpstead R, O'Brien M, Lee A and Meltzer H (2001) Psychiatric Morbidity among adults living in private households, 2000, TSO: London.

The correct answer is: DSH without suicidal intent → 2%, Suicide attempts → 4%, Suicidal thoughts → 15%

Question 61

Not answered

Marked out of 5.00

Motor Symptoms

Choose the most appropriate catatonic phenomenon from the explanations given below:

The maintenance of strange and uncomfortable limb and body positions.

Choose...



The patient carries out every command in a literal, concrete fashion, despite requests to resist manipulation

Choose...



The patient resists carrying out the examiner's instructions and his attempts to move or direct the limbs.

Choose...



The patient is lying supine on the bed but his head is not in contact with the bed surface

Choose...



Repetition of words or phrases, while unable to articulate the next word in the sentence by the patient

Choose...



Your answer is incorrect.

Explanation:

Posturing refers to maintenance of odd and bizarre postures. These might be spontaneously undertaken or derived from an arrested motor activity e.g. posture with swung arms as if one is frozen when walking. This is maintained despite efforts to be moved. It is also called catalepsy.

Automatic obedience: Exaggerated cooperation with examiner's request or spontaneous continuation of movement requested. To demonstrate this, the examiner must ask the patient not to cooperate; but still the patient will carry out motor instructions.

Negativism: Patients with negativism resist or oppose all passive movements attempted by the examiner. A mild form of such resistance is called Gegenhalten or opposition. In extreme forms it is called negativism where apparently motiveless resistance to all interference is found.

Psychological Pillow: This is an extreme form of posturing. The patient holds their head several inches above the bed while lying and can maintain this uncomfortable posture for long periods of time.

Verbigeration: Repetition of words or phrases, while unable to articulate the next appropriate word in a sentence. Often seen in expressive dysphasia.

The correct answer is: The maintenance of strange and uncomfortable limb and body positions. → Posturing, The patient carries out every command in a literal, concrete fashion, despite requests to resist manipulation → Automatic obedience, The patient resists carrying out the examiner's instructions and his attempts to move or direct the limbs. → Negativism, The patient is lying supine on the bed but his head is not in contact with the bed surface → Psychological pillow, Repetition of words or phrases, while unable to articulate the next word in the sentence by the patient → Verbigeration



Question 62

Not answered

Marked out of 5.00

Agitation & Psychiatric Disorders (1)

The following conditions or disorders are associated with agitation. Please match the following condition or disorder with the most likely clinical scenario.

A 28-year-old woman is apprehended by the Police on the high street at 4am. She is in her underwear, jumping in and out of traffic and seemingly unaware of the danger she is posing to herself. When the Police question her they find it difficult to follow what she is saying: the rate and volume of her speech is increased. She is talking about being very rich and famous, and makes sexualised gestures to the male Police officer. She has not slept for the past four nights

Choose...

A 22-year-old man is shouting in the street. He is extremely angry, having just been involved in a fight in a pub. This was triggered by a disagreement with another man over the result of a football game. During the fight he smashed an empty bottle on the other man's head. He shows no remorse for his actions, insisting that he was right to act in the way he did. He has three previous convictions for aggravated bodily harm, and one for robbery

Choose...

An 83-year old man is found wandering in the street, two miles from his home. He does not know what street he's on, but he's able to give his name and recall the date and time. He says he went out to buy the paper and lost his way. As he talks there are a few words he struggles to find. He's able to give a clear account of the early part of his life

Choose...

A 45-year old man is found in the street shouting at people on a bus. He claims that the people on the bus are controlling his thoughts, that they are purposefully removing any good thoughts he has, and replacing them with evil ones. He is convinced that the IRA are behind this activity

Choose...

A 32-year old woman is found distressed and weeping at 3am on the high street. She says that her friends have left her to get home on her own. She is slurring her words and her movements are not well controlled. She is not able to walk in a straight line and reports feeling nauseated

Choose...

Your answer is incorrect.

Explanation:

Q1: This scenario is suggestive of a classic manic episode, with increased activity, pressure of speech, grandiosity, risk-taking, sexualised behaviour, and lack of sleep.

Q2: Features suggestive of Antisocial Personality Disorder include past history of aggressive and law-breaking behaviour, lack of remorse of violent behaviour, and low tolerance for frustration.

Q3: Dementia can present with wandering, but unlike delirium, it occurs in clear consciousness, and there is often good memory retention for historical events.

Q4: The given account of thought interference and delusions makes a diagnosis of a psychotic illness the most likely explanation.

Q5: The presence of emotional lability, nausea, slurred speech and unsteady gait suggest alcohol intoxication as a cause for this young woman's agitation.

The correct answer is: A 28-year-old woman is apprehended by the Police on the high street at 4am. She is in her underwear, jumping in and out of traffic and seemingly unaware of the danger she is posing to herself. When the Police question her they find it difficult to follow what she is saying: the rate and volume of her speech is increased. She is talking about being very rich and famous, and makes sexualised gestures to the male Police officer. She has not slept for the past four nights → Mania, A 22-year-old man is shouting in the street. He is extremely angry, having just been involved in a fight in a pub. This was triggered by a disagreement with another man over the result of a football game. During the fight he smashed an empty bottle on the other man's head. He shows no remorse for his actions, insisting that he was right to act in the way he did. He has three previous convictions for aggravated bodily harm, and one for robbery → Antisocial Personality Disorder, An 83-year old man is found wandering in the street, two miles from his home. He does not know what street he's on, but he's able to give his name and recall the date and time. He says he went out to buy the paper and lost his way. As he talks there are a few words he struggles to find. He's able to give a clear account of the early part of his life → Alzheimer's Dementia, A 45-year old man is found in the street shouting at people on a bus. He claims that the people on the bus are controlling his thoughts, that they are purposefully removing any good thoughts he has, and replacing them with evil ones. He is convinced that the IRA are behind this activity → Acute Psychotic disturbance, A 32-year old woman is found distressed and weeping at 3am on the high street. She says that her friends have left her to get home on her own. She is slurring her words and her movements are not well controlled. She is not able to walk in a straight line and reports feeling nauseated → Intoxication with Alcohol

Question 63

Not answered

Marked out of 4.00

Agitation & Psychiatric Disorders (1)

The following conditions or disorders are associated with agitation. Please match the following condition or disorder with the most likely clinical scenario.

An 18-year old man is walking on a public street at 5am. He moves very quickly, and his speech is increased in rate, with an emphatic quality. He claims to have been having a very enjoyable night and to have made friends of many strangers. Of note, his pupils are dilated. His energy levels have return to normal six hours later

Choose...

An 87-year old woman is found outside her house, crossing the road without due attention to the traffic. She is unsteady on her feet and disorientated to time, place and person. She is intermittently drowsy. Her speech is disorganised and she has been incontinent

Choose...

A 26-year old woman is walking down the street when suddenly her respiratory rate increases and she clutches her chest. She appears frightened and tremulous. This episode lasts a few minutes and resolves spontaneously

Choose...

A 72-year old man is waiting at a bus stop on the high street and cannot remain still. He paces backwards and forward and repeatedly rings his hands. He says that he's committed a terrible crime, having stolen some chewing gum from a shop when he was a teenager, and now he is destined to have a terrible life. He says that nothing can help him with the awful guilt he is experiencing, and that he deserves to die

Choose...

Your answer is incorrect.

Explanation:

Q1: Dilated pupils can occur with use of recreational stimulants such as **MDMA**, which increase energy levels, but whose effects wear off after some hours.

Q2: This is a picture of delirium, characterised by the clouding of consciousness, disorientation and physical impairment. Her incontinence may be suggestive of a UTI as a causative factor.

Q3: Panic attacks occur spontaneously and resolve within a few minutes. Rapid shallow breathing, chest tightness, tremors and extreme states of anxiety are features of a classic panic attack.

Q4: This man's thoughts are of depressive nature. Restlessness and agitation can sometimes accompany a depressive episode, especially in older people.

The correct answer is: An 18-year old man is walking on a public street at 5am. He moves very quickly, and his speech is increased in rate, with an emphatic quality. He claims to have been having a very enjoyable night and to have made friends of many strangers. Of note, his pupils are dilated. His energy levels have return to normal six hours later → Intoxication with MDMA, An 87-year old woman is found outside her house, crossing the road without due attention to the traffic. She is unsteady on her feet and disorientated to time, place and person. She is intermittently drowsy. Her speech is disorganised and she has been incontinent → Delirium, A 26-year old woman is walking down the street when suddenly her respiratory rate increases and she clutches her chest. She appears frightened and tremulous. This episode lasts a few minutes and resolves spontaneously → Panic attack, A 72-year old man is waiting at a bus stop on the high street and cannot remain still. He paces backwards and forward and repeatedly rings his hands. He says that he's committed a terrible crime, having stolen some chewing gum from a shop when he was a teenager, and now he is destined to have a terrible life. He says that nothing can help him with the awful guilt he is experiencing, and that he deserves to die → Agitated depression



Question 64

Not answered

Marked out of 3.00

Catatonia (1)

Please match the scenarios described with the most likely cause for catatonia.

A 66-year old man is admitted to hospital after he stopped eating and drinking two days previously. When you attempt to interview the patient he is immobile on his bed, and will not engage you in conversation nor make eye contact. His wife tells you that he had been complaining of low energy and feeling worthless for the past three weeks

Choose...

You visit a 37-year old man at his home after his family called mental health services expressing their concerns about him. He is standing up, facing the wall of his bedroom and has not moved for 5 hours. He is staring ahead and does not turn his head to speak with you. He missed a Haldol depot two weeks previously.

Choose...

A 67-year old woman is found almost immobile in her bed by a relative. She had been complaining of a persistent headache for the preceding few weeks, which was worse when stooping down. Her clothes appear loose and ill-fitting.

Choose...

Your answer is incorrect.

Explanation: The account of preceding depressive symptoms (low energy and worthlessness) suggest this man's stupor is related to a depressive illness. The stupor experienced by this man is likely to be related to a relapse in his psychotic illness following non-compliance of antipsychotic depot medication. Persistent headache (worse on stooping down), and weight loss suggest a space-occupying lesion as a potential cause for this woman's stupor.

The correct answer is: A 66-year old man is admitted to hospital after he stopped eating and drinking two days previously. When you attempt to interview the patient he is immobile on his bed, and will not engage you in conversation nor make eye contact. His wife tells you that he had been complaining of low energy and feeling worthless for the past three weeks → Depressive Stupor, You visit a 37-year old man at his home after his family called mental health services expressing their concerns about him. He is standing up, facing the wall of his bedroom and has not moved for 5 hours. He is staring ahead and does not turn his head to speak with you. He missed a Haldol depot two weeks previously. → Schizophrenia, A 67-year old woman is found almost immobile in her bed by a relative. She had been complaining of a

persistent headache for the preceding few weeks, which was worse when stooping down. Her clothes appear loose and ill-fitting. → Intracranial space-occupying lesion



Question 65

Not answered

Marked out of 3.00

Catatonia (2)

Please match the scenarios described with the most likely cause for catatonia.

A 42-year old man is changed from an oral antipsychotic, to a depot injection. A week after the depot he complains of stiffness in his arms, and the following day he lies rigidly in bed, sweating profusely, with his eyes moving around and not able to rest on your face as you attempt to talk to him

Choose...

A 17-year old young man has been brought onto the ward after an aggressive outburst at home. He is now standing very still, with his torso leaning forwards, and has kept this position for two hours. You try to engage him in conversation, and his father tells you that he does not communicate verbally, and never has done

Choose...

A 22 -year old woman is brought to hospital by her friends after she stopped moving several hours previously. They tell you that for the past three days she has been out of the house all day and all night, that she reported feeling completely wonderful, and had spent all her savings on unusual jewellery

Choose...

Your answer is incorrect.

Explanation:

Q1: This man is now presenting with features of Neuroleptic malignant syndrome, which includes muscular rigidity, hyperthermia and diaphoresis. Administering a depot injection is considered as one of the risk factors for developing NMS.

Q2: A young man who does not have verbal speech and who has had an aggressive outburst may suggest he has Autism. People with Autism can sometimes enter catatonic states or maintain postures for long periods of time.

Q3: The history preceding the stupor is suggestive of mania (elated mood, excessive spending, lack of sleep/increased energy).

The correct answer is: A 42-year old man is changed from an oral antipsychotic, to a depot injection. A week after the depot he complains of stiffness in his arms, and the following day he lies rigidly in bed, sweating profusely, with his eyes moving around and not able to rest on

your face as you attempt to talk to him → Neuroleptic Malignant Syndrome, A 17-year old young man has been brought onto the ward after an aggressive outburst at home. He is now standing very still, with his torso leaning forwards, and has kept this position for two hours. You try to engage him in conversation, and his father tells you that he does not communicate verbally, and never has done → Autism, A 22 -year old woman is brought to hospital by her friends after she stopped moving several hours previously. They tell you that for the past three days she has been out of the house all day and all night, that she reported feeling completely wonderful, and had spent all her savings on unusual jewellery → Manic stupor



Question 66

Not answered

Marked out of 3.00

Starvation & Psychiatric emergency (1)

A starving patient can present with a psychiatric emergency. Please match the scenarios below with the most likely cause of starvation.

A 24-year old man has lost 16 kilos over one month by restricting his food intake. He is noted to obsessively estimate the calories of his meals, and has increased his exercise regime to help his lose further weight. He wishes to be thinner than he is currently

Choose...

An 18-year old woman has lost several kilos over the past week. She has been constantly active, has not been sleeping, and has been dedicating herself to a project of redesigning the London underground system, a commission she claims she was given by the Prime Minister

Choose...

A 49-year old man is found to have lost 10% of his body weight over the past 3 months. When you visit him at home you find him in a dishevelled and unwashed state. He cannot hold a conversation as he is distracted, and he seems to be whispering to himself

Choose...

Your answer is incorrect.

Explanation: This scenario is indicative of an eating disorder, likely anorexia nervosa, characterised by food restriction and exercise in order to lose weight, a wish to be thin and calorie-counting. This young woman is neglecting her basic needs (eating regular meals) due to her manic illness, as suggested by her increased energy, lack of sleep, and grandiosity. A psychotic illness can be associated with starvation and self-neglect.

The correct answer is: A 24-year old man has lost 16 kilos over one month by restricting his food intake. He is noted to obsessively estimate the calories of his meals, and has increased his exercise regime to help his lose further weight. He wishes to be thinner than he is currently → Eating disorder, An 18-year old woman has lost several kilos over the past week. She has been constantly active, has not been sleeping, and has been dedicating herself to a project of redesigning the London underground system, a commission she claims she was given by the Prime Minister → Manic neglect, A 49-year old man is found to have lost 10% of his body weight over the past 3 months. When you visit him at home you find him in a dishevelled and unwashed state. He cannot hold a conversation as he is distracted, and he seems to be whispering to himself → Schizophrenia

Question 67

Not answered

Marked out of 3.00

Starvation & Psychiatric emergencies (2)

A starving patient should be considered as a potential psychiatric emergency. Match the scenarios below with the most probable cause of starvation.

A 45-year old woman has been eating under 500 calories of food a day. She tells you that most of what she prepares has to be thrown away as it becomes contaminated by bacteria on kitchen surfaces. Her kitchen appears spotless and you notice her hands are severely excoriated from excessive washing

Choose...

A 62-year old man has lost 12 kilos of weight over the previous three weeks. He has been eating very little as he says he doesn't deserve to eat: the devil is awaiting him in hell to punish his for the evil he has committed. He hasn't experienced any enjoyment in life and hasn't seen any point in being alive for over a month now

Choose...

A 30-year old man hasn't eaten for two days now. He claims to be feeling unwell, is noted to have tremor and his temperature is elevated. There has recently been a change in his medication

Choose...

Your answer is incorrect.

Explanation:

Case 1: This vignette is suggestive of an obsessive illness, in this case related to a fear of contamination.

Case 2: This man has a psychotic depressive illness: lack of enjoyment, worthlessness, suicidality and depressive delusions.

Case 3: Tremor, elevated temperature in the context of a change in medication suggests a phenomenon such as neuroleptic malignant syndrome or serotonin syndrome. This is a medical emergency, and should be treated with supportive measures, and the offending medication should be stopped.

The correct answer is: A 45-year old woman has been eating under 500 calories of food a day. She tells you that most of what she prepares has to be thrown away as it becomes contaminated by bacteria on kitchen surfaces. Her kitchen appears spotless and you notice her hands are severely excoriated from excessive washing → OCD with food-related obsessions, A 62-year old man has lost 12 kilos of weight over the

previous three weeks. He has been eating very little as he says he doesn't deserve to eat: the devil is awaiting him in hell to punish his for the evil he has committed. He hasn't experienced any enjoyment in life and hasn't seen any point in being alive for over a month now → Psychotic depression, A 30-year old man hasn't eaten for two days now. He claims to be feeling unwell, is noted to have tremor and his temperature is elevated. There has recently been a change in his medication → Psychotropic-related starvation



Question 68

Not answered

Marked out of 5.00

Suicide demographics

Please match the most likely demographic picture with each patient suicide scenarios described below.

This patient died as a result of inhalation of car exhaust fumes

Choose...



This patient took their life by taking an overdose of prescribed medication

Choose...



Death in this patient occurred due to a morphine overdose

Choose...



This patient died by taking an overdose of over-the-counter medication. Lacerations were noted on the patient's arms

Choose...



This patient died by hanging

Choose...



Your answer is incorrect.

Explanations: The principal methods of suicide in men aged 15 to 19 years is poisoning with car exhaust fumes and hanging. In most countries the highest rate of suicide is among people aged over 75. In women the most frequent method is drug overdose. Dentists, doctors and pharmacists are at increased risk of suicide due to access to medications, such as controlled drugs. The commonest method of suicide in women is drug overdose. Self-harm by laceration is relatively common in women with borderline personality disorder, 10% of whom eventually take their own life. Hanging accounts for almost 40% of deaths among men. Those who are unemployed and single/divorced are at highest risk.

The correct answer is: This patient died as a result of inhalation of car exhaust fumes → 18-year old student, This patient took their life by taking an overdose of prescribed medication → 78-year old widowed woman, Death in this patient occurred due to a morphine overdose → 47-year old male dentist, This patient died by taking an overdose of over-the-counter medication. Lacerations were noted on the patient's arms → 22-year old single woman, This patient died by hanging → 45 year old unemployed, divorced man

Question 69

Not answered

Marked out of 5.00

Treatments in Emergencies

This question concerns urgent psychiatric scenarios. Match the most appropriate treatment with each scenario given.

Ensure the airway is protected, fluid balance is maintained and the potentially causative medication is stopped

Choose...



De-escalate the situation by non-medication means first

Choose...



Minimise potential risk of harm and consider administration of Lorazepam 1mg

Choose...



Use supportive measures including IV Fluids and investigate potential causes

Choose...



Re-introduce the medication or choose one with a longer half-life

Choose...



Your answer is incorrect.

Explanation:

The management of Neuroleptic Malignant Syndrome is mainly supportive. In severe cases a patient may need to be ventilated and treated in ITU; dopaminergic drugs (such as bromocriptine or amantadine) may be given. The offending drug should be stopped immediately.

In cases of violence or aggression it is best practice to use non-medication means of de-escalating the situation. A change in environment and nursing staff interventions may suffice. In cases where a patient may put themselves at risk of harm due to severe agitation, consider means of reducing risk in the immediate environment. Benzodiazepines such as short-acting Lorazepam may be given orally, or if necessary to reduce risk of harm, intramuscularly.

Delirium is a medical emergency and should be adequately treated via supportive measures while the potential cause (such as an infection) can be investigated and subsequently treated.

Discontinuation syndrome for antidepressants may include symptoms such as flu-like symptoms, sleep disturbance, sensory disturbance, mood and cognitive disturbance. It can occur if an antidepressant has been stopped too abruptly. Re-starting the antidepressant that was discontinued should bring some relief. The use of an antidepressant with a long half life (such as Fluoxetine) may be useful in some cases.

The correct answer is: Ensure the airway is protected, fluid balance is maintained and the potentially causative medication is stopped → Neuroleptic Malignant Syndrome, De-escalate the situation by non-medication means first → Aggressive outburst involving violence between patients, Minimise potential risk of harm and consider administration of Lorazepam 1mg → Severe agitation in the context of auditory hallucinations and delusions, Use supportive measures including IV Fluids and investigate potential causes → Delirium, Re-introduce the medication or choose one with a longer half-life → Antidepressant Discontinuation Syndrome



Question 70

Not answered

Marked out of 6.00

Antipsychotic doses

For each of the following antipsychotics, identify the UK licensed maximum doses of antipsychotics that could be prescribed for oral administration

Haloperidol	Choose...
Chlorpromazine	Choose...
Olanzapine	Choose...
Quetiapine	Choose...
Risperidone	Choose...
Clozapine	Choose...

Your answer is incorrect.

Explanation: The UK licensed maximum doses of antipsychotics that could be prescribed for oral administration include Haloperidol- 20mg/day (from 2014), Chlorpromazine- 1000mg/day, Olanzapine- 20mg/day, Quetiapine- 750-800 mg/day, Risperidone- 16 mg/day, Clozapine- 900 mg/day, Amisulpride- 1200 mg/day, Aripiprazole-30 mg/day, Paliperidone- 12 mg/day. Doses above those maxima should only be used in extreme circumstances, as there is no evidence for improved efficacy.

In 2014, the British National Formulary (BNF) reduced its maximum daily IM dose for haloperidol from 18mg to 12mg and the oral dose from 30 to 20mg. This was done to reflect changes in the IM Haldol product literature. The bioavailability for the oral route is nearly 60% of that of the IM route.

Ref: Maudsley prescribing guidelines- Pg 3 & The BNF (v. 67 onwards)

The correct answer is: Haloperidol → 20 mg/day, Chlorpromazine → 1000mg/day, Olanzapine → 20 mg/day, Quetiapine → 750-800 mg/day, Risperidone → 16 mg/day, Clozapine → 900 mg/day

Question 71

Not answered

Marked out of 6.00

Antidepressant doses

For each of the following antidepressants, identify the UK licensed maximum doses that could be prescribed for treatment of depression

Mirtazapine	Choose... ▾
Venlafaxine	Choose... ▾
Duloxetine	Choose... ▾
Citalopram	Choose... ▾
Sertraline	Choose... ▾
Escitalopram	Choose... ▾

Your answer is incorrect.

Explanation: The UK licensed maximum doses of antidepressants that could be prescribed for treatment of depression include Mirtazapine- 15-45 mg/day, Venlafaxine 75-375 mg/day, Duloxetine 60-120 mg/day, Citalopram- 20-60 mg/day, Fluoxetine 20-60 mg/day, Fluvoxamine 100-300 mg/day, Escitalopram 10-20 mg/day, Paroxetine 20-50 mg/day, Sertraline 50-200 mg/day, Amitriptyline 30-200 mg/day, Dothiepin 75-225 mg/day, Imipramine 10-200 mg/day and Lofepramine 140-210 mg/day. (Ref: Maudsley prescribing guidelines- Pg 150-160)

The correct answer is: Mirtazapine → 15-45 mg/day, Venlafaxine → 75-375 mg/day, Duloxetine → 60-120 mg/day, Citalopram → 20-60 mg/day, Sertraline → 50-200 mg/day, Escitalopram → 10-20 mg/day

Question 72

Not answered

Marked out of 5.00

Eponymous syndromes (1)

Find one eponym each for the syndromes described below:

A 45-year-old gentleman lost his vision following bilateral occipital cortex damage. However he insists that he has a normal vision

Choose...

A 67-year-old woman admitted to the psychiatric ward is convinced that she is already dead, and should be buried, that her insides have stopped working and are rotting away.

Choose...

A 28-year-old secretary believes that the British Prime Minister David Cameron is in love with her, although she has no evidence to prove it.

Choose...

Twin sisters aged 63 and 61; living together expressed the identical conviction that aliens are breaking into their property at night-time

Choose...

A 45 year-old-gentleman believes that his wife is having an affair with another man. He checks all her personal belongings looking for evidence and argues with her constantly.

Choose...

Your answer is incorrect.

Explanation: **Anton syndrome** is a rare symptom of brain damage occurring in the occipital lobe. People who suffer from it are 'cortically blind' but affirm, often quite adamantly and in the face of clear evidence of their blindness, that they are capable of seeing. Failing to accept being blind gets dismissed by the sufferer through confabulation. **Cotard's syndrome** is a presentation of psychotic depressive illness seen particularly in elderly people. There is a combination of severely depressed mood with nihilistic delusions and/ or hypochondriacal delusions. The patient may state that he is already dead and should be buried, that his insides have stopped working and are rotting away, or that he has stopped existing altogether. **de Clerambault's syndrome** is a form of delusion of love. The patient, usually female, believes that another, higher-status individual is in love with them. The object may be an employer or doctor, or in some cases a prominent public figure or celebrity. **Folie a deux** describes a situation where two people with a close relationship share a delusional belief. This arises as a result of a delusional belief, which comes to be shared by the second. The delusion resolves in the second person on separation, the first

should be assessed and treated in the usual way. **Delusional jealousy:** A delusional belief that one's partner is being unfaithful. This can occur as part of a wider psychotic illness, secondary to organic brain damage associated with alcohol dependence, or as a monosymptomatic delusional disorder (Othello syndrome) (Ref: Oxford handbook of psychiatry; pg 80-100)

The correct answer is: A 45-year-old gentleman lost his vision following bilateral occipital cortex damage. However he insists that he has a normal vision → Anton's syndrome, A 67-year-old woman admitted to the psychiatric ward is convinced that she is already dead, and should be buried, that her insides have stopped working and are rotting away. → Cotard's syndrome, A 28-year-old secretary believes that the British Prime Minister David Cameron is in love with her, although she has no evidence to prove it. → De Clerembault syndrome, Twin sisters aged 63 and 61; living together expressed the identical conviction that aliens are breaking into their property at night-time → Folie a Deux, A 45 year-old-gentleman believes that his wife is having an affair with another man. He checks all her personal belongings looking for evidence and argues with her constantly. → Othello syndrome



Question 73

Not answered

Marked out of 6.00

High risk prediction in schizophrenia

A population of high-risk adolescents with some symptoms suggestive of psychosis is followed up for 12 months. Choose the most appropriate answers for the following questions from the given list:

1. Which is the most important predicting factor for full psychosis? (ONE) ✖

2. What are 2 other predicting factors? (TWO) ✖

✖

3. Which are protective factors against psychosis? (TWO) ✖

✖

4. Which neuropsychological deficit is most discriminatory? (ONE) ✖

Explanation:

The NAPLS - North American Prodrome Longitudinal study: This is a blend of several individual North American cohorts representing a pragmatic approximation of a prospectively designed cohort study. The key findings are that the ultrahigh risk UHR criteria using SIPS (Structured interview for prodromal symptoms) predict an early transition to psychosis (with a huge relative risk of 405 compared with the incident rate of psychotic disorders in the general population). In this group the predictive power can probably be enhanced by the use of key variables such as genetic risk, functional impairment, and higher levels of psychopathology at baseline (especially odd beliefs and suspiciousness). Substance use also increased the predictive value, but the former 3 variables together nearly doubled the predictive power. In multivariate analyses no specific substance class of the 7 tested (ie, alcohol, hypnotics, cannabis, amphetamines, opiates, cocaine, and hallucinogens) was significantly associated with risk though as a whole a history of substance use was predictive.

Edinburgh High Risk Study: It is thought that the individuals at enhanced genetic risk of schizophrenia (family history) inherit a state of vulnerability characterised by transient and partial psychosis like symptoms; but not all of these develop floridschizophrenia. Measures of episodic memory are also proposed to be significantly discriminating between those with high risk who develop schizophrenia from those

who do not; this is suggestive of temporal lobe dysfunction.

(Ref: Cannon TD, et al. Prediction of psychosis in youth at high clinical risk: a multisite longitudinal study in North America. Arch Gen Psychiatry. 2008;65(1):28-37. , Johnstone EC, et al. Predicting schizophrenia-findings from the Edinburgh High-Risk Study.Br J Psychiatry. 2005;186:18-25)

Correct Answer is : 1. Genetic history in the relatives 2. Poor functioning (GAF score), Long duration of prodromal symptoms 3. Antipsychotics for relapse prevention, Education about early relapse signs 4. Episodic memory deficits



Question 74

Not answered

Marked out of 6.00

Clinical features (2)

Choose 2 characteristic features for each of the diagnosis given below:

1. Anorexia nervosa (restricting type) (TWO)
2. Bulimia nervosa (TWO)
3. Post traumatic stress disorder (TWO)

Explanation:

One of the diagnostic features of anorexia nervosa is the presence of endocrine disturbances involving the hypothalamic-pituitary-gonadal axis that manifests in women as amenorrhoea and in men as a loss of sexual interest and potency. There may also be elevated levels of growth hormone, raised levels of cortisol, changes in the peripheral metabolism of the thyroid hormone, and abnormalities of insulin secretion.

In bulimia, a diagnostic feature is that the patient attempts to counteract the "fattening" effects of food by one or more of the following: self-induced vomiting; purgative abuse, alternating periods of starvation; use of drugs such as appetite suppressants, thyroid preparations or diuretics.

Both ICD-10 and DSM-5 diagnostic criteria for PTSD include a history of exposure to a traumatic event and symptoms from three (ICD) or four (DSM-5) symptom clusters. The symptom clusters include intrusive recollections, avoidant symptoms, negative cognition and mood (DSM-5: includes self-blame, blaming others, negative emotional state expressed sometimes as irritability) and hyper-arousal symptoms. Both ICD and DSM also include a criterion concerning duration of symptoms.

Correct Answer is : 1. Amenorrhea, Loss of sexual drive 2. Self-purging, Self induced vomiting 3. Hyperarousal symptoms, Irritability

Question 75

Not answered

Marked out of 6.00

Rating scales (1)

Identify the relevant rating scales for each of the following conditions

1. Choose TWO observer rated tests to monitor depression in a woman recently diagnosed with postnatal depression

2. Choose the most appropriate tool to screen for depression in mothers following childbirth

3. Choose one scale to assess EPSEs due to antipsychotic use.

4. Choose one scale which retains the features of a clinical examination

5. Choose one scale which is a screening instrument for the presence of psychiatric caseness

Explanation:

Hamilton depression scale and Montgomery Asberg depression rating scale are observer rated test used to monitor depression.

The Edinburgh Postnatal Depression Scale (EPDS) have been used to screen for depression in women during the antepartum and postpartum periods. Edinburgh Postnatal Depression Scale was specifically developed for assessing postpartum depression and relies much less on somatic questions. Routine use of EPDS during the postpartum period has been shown to increase the detection of postpartum depression compared with usual care.

The Simpson Angus scale is helpful to assess EPSEs due to the use of antipsychotic medications. Present state examination retains the features of a clinical examination. It is clinician administered semi-structured clinical interview, which provides clinical diagnosis.

General Health Questionnaire is a self-rated screening instrument for presence of psychiatric illness.

Correct Answer is : 1. Hamilton Depression scale, Montgomery Asberg Depression Rating scale 2. Edinburg postnatal depression scale 3. Simpson Angus scale 4. Present state examination 5. General Health Questionnaire



Question 76

Not answered

Marked out of 10.00

Prognosis in schizophrenia

Mr. P has been diagnosed with schizophrenia and his father is keen to know about prognostic options. He is keen to know the answers for the following questions. Please choose the most appropriate responses from the drop-down lists.

1. Choose two factors predicting good prognosis in short term  
  
2. Choose two factors predicting good prognosis in long term  
  
3. Choose two factors predicting poor prognosis in short term  
  
4. Choose two factors predicting poor prognosis in long term  
  
5. Choose two factors that is not associated with prognostic significance in schizophrenia
    

Explanation:

In longer term, poor prognosis is predicted by prominent negative (deficit) symptoms, longer duration of initial episode (refer to ISoS data reappraisal by Harrison 2001) and longer duration of untreated psychosis. The issue of family history is unresolved - there is some evidence that family loading of mood disorders may suggest favourable prognosis though this has not been consistently shown. The evidence for poor

prognosis in those with positive family history of schizophrenia (non-affective psychosis) is sparse. Age of onset is also controversial when it comes to longer term prediction.

Correct Answer is : 1. Positive response to placebo, Good initial response to antipsychotics 2. Being female, Acute onset of symptoms 3. Daily use of cannabis, Poor compliance with treatment 4. Long duration of untreated psychosis, Male gender 5. Family history of schizophrenia (1st degree relative), Young age of onset



Question 77

Not answered

Marked out of 6.00

Psychiatric rating scales

Identify the most appropriate rating scale for the following conditions;

1. Mrs.X suffers from OCD. She has recently delivered a baby. The health visitor would like to screen her for depression

  

2. Mr.Y is taking high dose of haloperidol. You wish to monitor the extent of parkinsonian side effects on him

  3. Mr. Z is a new trainee in psychiatry who would like to interview a patient in a standardised fashion but still retaining the features of a clinical examination  

4. Mr. A is a GP registrar who would like to use a computer programme to generate psychiatric diagnosis

  5. Mrs. P has been diagnosed with postnatal depression and has been commenced on antidepressants. You wish to monitor her response to treatment (Choose TWO)    **Explanation:**

Edinburgh postnatal depression scale is a screening tool used to identify depression in mothers following childbirth. Simpson Angus scale is helpful to assess the extent of extrapyramidal side effects caused by antipsychotic medications such as haloperidol.

Present state examination retains the features of a clinical examination and is helpful in interviewing patients.

CATEGO is a computer programme for processing data from the schedules of clinical assessments in neuropsychiatry.

Montgomery & Asberg depression rating scale and Hamilton rating scale are useful to monitor response to treatment with antidepressant medications.

Correct Answer is : 1. Edinburgh postnatal depression scale 2. Simpson-Angus scale 3. Present state examination 4. CATEGO 5. Montgomery and Asberg depression rating scale, Hamilton depression rating scale



Question 78

Not answered

Marked out of 8.00

Diagnostic features

For each of the following diagnosis choose the most characteristic symptoms.

1. Paranoid schizophrenia (TWO)

  
  

2. Persistent delusional disorder (THREE)

  
    

3. Hebephrenic schizophrenia (THREE)

  
    
Explanation:

Prominent delusions and hallucinations are the most prominent features of paranoid schizophrenia. Other symptoms listed here are mostly supplementary. Though an age criterion is not required for a diagnosis (unlike hebephrenia), it is a well-known clinical observation that paranoid schizophrenia has later age of onset compared to hebephrenia or catatonia.

Persistent delusional disorder is characterized by later age of onset (mean 40yrs) and symptoms must be present for more than 3 months for a diagnosis to be made (according to ICD-10). Somewhat preserved social functioning is noted compared to schizophrenia (according to DSM).

Hebephrenic schizophrenia is characterized by emotional disturbances (incongruence or a fatuous affect), abnormal premorbid personality and younger age of onset

Correct Answer is : 1. Comparatively later age of onset, Prominent delusions and hallucinations 2. Comparatively later age of onset, Symptoms last for at least 3 months, Preserved social and occupational functioning 3. Affective disturbance, Pre-morbid schizoid

personality, Young adult or adolescent

Question 79

Not answered

Marked out of 7.00

Treatment of OCD

Identify treatment options for OCD in adults according to the most recent NICE guidelines

1. OCD with mild functional impairment ✖

2. OCD with moderate functional impairment (choose 2 answers) ✖

✖

3. OCD with severe functional impairment with unsuccessful first line therapy ✖

4. Treatment resistant OCD (choose 3 answers) ✖ ✖

✖ ✖

Explanation: For OCD with mild functional impairment, brief CBT (exposure and response prevention) with less than 10 therapist hours should be tried. OCD with moderate functional impairment-offer choice of more intensive CBT for more than 10 therapist hours or course of SSRI. For OCD with severe functional impairment- offer combined treatment of CBT and SSRI. For treatment resistant OCD, refer to multidisciplinary team with expertise in OCD, consider additional CBT, or adding antipsychotic to SSRI or using clomipramine, or combining clomipramine and citalopram.

Correct Answer is : 1. Brief CBT 2. Course of SSRI, Intensive CBT 3. Combined treatment of SSRI and CBT 4. Adding antipsychotic to SSRI, Adding clomipramine, Refer to a team with expertise in OCD

Question 80

Not answered

Marked out of 6.00

Prophylaxis in bipolar disorder

Choose the best option for the following clinical situations

1. The first line prophylactic agents in bipolar disorder (choose THREE)  
    
2. The class of drugs which should be used in combination with a mood stabilizer to treat acute episodes of depression but should not be used routinely for prophylaxis  
3. Prophylaxis against frequent episodes of bipolar depression rather than mania  
4. Associated with a reduced risk of suicide and effective against both manic and depressive relapse 


Explanation: Lithium, valproate and olanzapine are the first line prophylactic agents in bipolar disorder (quetiapine can now be added to this list). SSRIs should be used in combination with a mood stabilizer to treat acute episodes of depression but should not be used routinely for prophylaxis. Combined Lithium and valproate could be used for the prophylaxis of rapid cycling illness. Lamotrigine appears to be effective both as a treatment of bipolar depression and as prophylaxis against further episodes of bipolar depression. Lithium is associated with a reduced risk of suicide and effective against both manic and depressive relapse

Correct Answer is : 1. Olanzapine, Valproate, Lithium 2. SSRIs 3. Lamotrigine 4. Lithium

Question 81

Not answered

Marked out of 13.00

Treatment of acute mania/hypomania

You are working in a busy affective disorders unit. There were 5 patients admitted with relapse of bipolar disorder. Choose the best course of action for each of the following scenario:

1. A 45-year-old woman with a history of bipolar disorder presents with elated mood, excessive energy and disturbed behaviour. She is on venlafaxine 225 mg (Choose TWO)

2. A 36-year-old woman with a history of bipolar disorder is suffering from a relapse of manic symptoms. She is currently on olanzapine (Choose THREE)

3. A 56-year-old man with a history of bipolar disorder is suffering from a relapse of manic symptoms. He is currently on Lithium carbonate 400 mg. (Choose FOUR)

4. A 39-year-old man with a history of bipolar disorder is suffering from a relapse of manic symptoms. He is currently on sodium valproate 800 mg. (Choose FOUR)

Explanation:

Q1: The diagnosis is antidepressant-induced mania. Therefore stop antidepressant treatment immediately. If symptoms are severe or behaviour is disturbed, consider adding an antipsychotic medication.

Q2: If patient is taking an antipsychotic and has still relapsed, then check compliance, check dose and increase if necessary (optimisation) and consider adding lithium or valproate.

Q3: If patient is taking Lithium as prophylaxis and has still relapsed, then check compliance, check dose & plasma levels and increase the dose to give levels of 1.0-1.2 m mol/L (optimisation) and consider adding an antipsychotic.

Q4: If a patient is taking sodium valproate as prophylaxis and has still relapsed, then check compliance, check dose & plasma levels and increase the dose to give levels of 125 mg/L (optimisation) and consider adding antipsychotic medication.

Correct Answer is : 1. Stop antidepressant treatment, Consider adding an antipsychotic 2. Check compliance, Check dose and increase if necessary, Consider adding lithium or valproate 3. Check compliance, Check dose and increase if necessary, Check plasma levels, Consider adding an antipsychotic 4. Check compliance, Check dose and increase if necessary, Check plasma levels, Consider adding an antipsychotic



Question 82

Not answered

Marked out of 6.00

Swapping antidepressants

For the clinical situations given below, choose the best option

1. A 45-year-old woman on paroxetine for 4 weeks has now decided to take citalopram, as she responded well to this medication in the past.

(Choose ONE)

  

2. A 52-year-old woman on Venlafaxine developed insomnia & dizzy spells. She is not keen to continue taking this antidepressant. She is now willing to take mirtazapine. (Choose ONE)

  

3. A 57-year-old woman on fluoxetine has atypical depression. Now decided to treat her on moclobemide. (Choose TWO)

    

4. A 46 year old man with depression on tranylcypromine has decided to switch over to sertraline (choose TWO)

    **Explanation:**

When swapping from one antidepressant to another, abrupt withdrawal should usually be avoided. Cross tapering is preferred, where the dose of the ineffective or poorly tolerated drug is slowly reduced while the new drug is slowly introduced.

When you switch from fluoxetine to moclobemide, withdraw and wait at least 5 weeks, as fluoxetine has the longest half-life of all SSRIs. When you switch from MAOI like tranylcypromine to SSRI, withdraw and wait for 2 weeks. MAOIs should not be co-administered with SSRIs, as this combination could precipitate serotonin syndrome.

Correct Answer is : 1. Cross taper with caution 2. Cross taper with caution 3. Withdraw and wait 5 weeks before starting new drug, Do not co-administer 4. Withdraw and wait 2 weeks before starting new drug, Do not co-administer

Question 83

Not answered

Marked out of 7.00

Antipsychotics & suitability

A medical student has heard about antipsychotic medications and their side effects. He is keen to discuss options with you about suitable drugs of choice for patients with co-morbid medical conditions.

1. Which of the 2 drugs could be suggested as antipsychotic agents to treat psychosis in patients with dyslipidemia/glucose intolerance?

   

2. Which 2 drugs are least likely to cause sexual side effects?    

3. Which drug is least likely to cause QT prolongation?  

4. Which 2 drugs are least likely to cause hyperprolactinaemia?    

Explanation: The suggested drugs for patients with dyslipidemias/impaired glucose tolerance are amisulpride, aripiprazole and ziprasidone. The suggested drugs for patients with sexual dysfunction are Aripiprazole and Quetiapine. Aripiprazole is least likely to cause QT prolongation. The suggested drugs for patients with hyperprolactinaemia are Aripiprazole and Quetiapine. (Ref: Maudlsey guidelines- 10th edition: pg 24)

Correct Answer is : 1. Aripiprazole, Amisulpride 2. Quetiapine, Aripiprazole 3. Aripiprazole 4. Quetiapine, Aripiprazole

Question 84

Not answered

Marked out of 7.00

Remedial measures in rapid tranquillization

Mr. Y is a 45-year-old gentleman admitted to psychiatric intensive care unit for agitation and extremely disturbed behaviour. He was given 1 mg of lorazepam and 5 mgs of haloperidol intramuscularly. For each of the following problems, identify the important action/s to be taken.

1. Increased temperature (Choose ONE) ✖
2. Fall in blood pressure by more than 30 mm Hg (Choose THREE) ✖
 ✖ ✖
3. Acute dystonia (Choose ONE) ✖
4. Irregular or slow pulse (less than 50/minute) (Choose ONE) ✖
5. Benzodiazepine induced respiratory depression (Choose ONE) ✖

Explanation:

Increased temperature following Rapid Tranquillization (RT) must prompt a blood test for creatinine kinase urgently, due to high risk of developing neuroleptic malignant syndrome.

Fall in blood pressure during RT must be considered significant if more than 30 mm Hg orthostatic drop occurs or if the diastolic BP is less than 50 mm Hg. In this case make the patient lie flat, tilt bed towards head and monitor closely.

Acute dystonia including oculogyric crisis can be managed by giving procyclidine 5-10mg IM or benztropine 1-2 mg IM.

Irregular or slow pulse (less than 50/minute) is a sinister sign; it necessitates a referral to specialist medical care immediately.

Reduced respiratory rate of less than 10/min or oxygen saturation less than 90% must be managed by giving oxygen, raising the legs, ensuring patient is not lying face down, and giving flumazenil if benzodiazepine induced respiratory depression suspected. If any other sedative agent induces such a respiratory arrest, transfer to a medical bed immediately and ventilate mechanically.

Correct Answer is : 1. Check creatinine kinase urgently 2. Tilt bed towards head, Make the patient to lie flat, Monitor blood pressure closely 3. Give procyclidine 5-10 mg IM 4. Refer to specialist medical care immediately 5. Give Flumazenil



Question 85

Not answered

Marked out of 7.00

Drug Treatments

Select the drug that could be used, for the treatment of each of the following situations;

1. Catatonia ✖
2. Neuroleptic malignant syndrome ✖ ✖
3. SIADH induced by antipsychotics ✖
4. Antidepressant induced hyponatraemia (2 answers) ✖ ✖
5. Wernicke's encephalopathy ✖

Explanation:

Catatonia is characterized by movement abnormalities usually associated with schizophrenia and mood disorders. Numerous studies and case reports indicate that benzodiazepines are rapidly effective, safe and easily administered and therefore the first line treatment for catatonia.

Neuroleptic malignant syndrome: Symptomatic management for vital signs instability, fluid replacement and prevention of renal failure are the main treatment methods after immediate stopping of the causative psychotropic agent. Dantrolene, bromocriptine or amantadine can be used in the management. Dantrolene is a peripheral muscle relaxant, which inhibits the intracellular calcium release from the sarcoplasmic reticulum. It was originally applied to treat cases of malignant hyperthermia.

For antipsychotic induced SIADH, if mild, fluid restriction with careful monitoring of serum sodium is suggested.

Antidepressant induced hyponatraemia can be managed by water restriction and prescribing non-SSRI agents, especially noradrenergic drugs such as reboxetine, lofepramine or an MAOI such as Moclobemide. Demeclocycline may be useful in recurrent cases of both SIADH or hyponatremia.

Wernicke's encephalopathy: Parenteral thiamine should be given as a treatment.

Correct Answer is : 1. Benzodiazepines 2. Dantrolene, Bromocriptine 3. Demeclocycline 4. Demeclocycline, Reboxetine 5. Thiamine



Question 86

Not answered

Marked out of 10.00

Management of EPSEs

Choose the most likely treatment options for the following types of EPSEs

1. Pseudo-parkinsonism caused by trifluoperazine (choose 3 answers)

2. Acute dystonia with painful arching of the back caused by administration of haloperidol

3. Akathisia induced by chlorpromazine (choose 3 answers)

4. Tardive dyskinesia induced by flupenthixol depot (choose 3 answers)

Explanation:

Reducing antipsychotic dose, changing to an atypical antipsychotic drug and prescribing an oral anticholinergic medication are the options to manage drug induced parkinsonism. Anticholinergic drugs may be given orally, IM or IV depending on the severity of symptoms.

In extreme cases of acute dystonia the back may arch or the jaw may dislocate. IM administration of anticholinergic drugs is the preferred option in this situation.

For akathisia, several options including reducing antipsychotic dose, changing to an atypical antipsychotic drug and prescribing propranolol 30-80 mg/day or low dose clonazepam can be used. Anticholinergics are generally unhelpful.

Reducing antipsychotic dose and changing to an atypical antipsychotic drug are possible options for Tardive dyskinesia. Clozapine is the antipsychotic most likely to be associated with resolution of symptoms. Stop any anticholinergic drugs previously prescribed.

Correct Answer is : 1. Reduce antipsychotic dose, Change to an atypical antipsychotic drug, Prescribe an oral anticholinergic 2. IM administration of anticholinergic drugs 3. Reduce antipsychotic dose, Change to an atypical antipsychotic drug, Propranolol 30-80 mg/day 4. Reduce antipsychotic dose, Change to an atypical antipsychotic drug, Consider clozapine



Question 87

Not answered

Marked out of 5.00

OCD

A 35-year-old gentleman has OCD. He is curious to know whether the type of ritual that he has will respond to treatment. Based on the descriptions given here, match his symptoms to the sub-types displayed on the list.

1. Which one symptom subtype confers a higher genetic risk on first-degree relatives? ❌

2. Which two types predict a poor response to SSRI antidepressants? ❌

❌

3. Which 2 types predict a poor response to CBT? ❌

❌

Explanation:

Symmetry/ordering rituals tend to be chronic and confers a higher genetic risk on first-degree relatives.

Sexual/religious obsessional thoughts and hoarding rituals predict a poor response to SSRIs and CBT.

Compulsive hoarding may be neurobiologically distinct form of obsessive-compulsive disorder. Hoarding is notoriously difficult to treat by either psychological or pharmacological means.

Correct Answer is : 1. Symmetry/ordering rituals 2. Sexual/religious obsessional thoughts, Hoarding rituals 3. Sexual/religious obsessional thoughts, Hoarding rituals

Question 88

Not answered

Marked out of 5.00

Sleep disturbances in PTSD

A 29-year-old soldier suffers from PTSD after returning from Iraq. His BMI is 24. He has had some improvement in PTSD with fluoxetine but continues to have sleep problems despite trials of zopiclone and temazepam. He admits taking nearly 6 units of alcohol every night. For each of the following description of his sleep problems, prescribe the most appropriate management from the given list:

1. He suffers from difficulty in falling asleep (initial insomnia) ✖
2. He continues to have terrible nightmares and finds it difficult to go back to sleep once he wakes up in the middle of the night. ✖
3. He suffers from daytime sleepiness and continues to have difficulty concentrating in his current job. ✖
4. His wife reports episodes of sudden breathlessness wherein he wakes up suddenly in the night sweating, with violent arm movements. (TWO options) ✖ ✖

Explanation:

70-91% of patients PTSD have difficulty falling or staying asleep. Additionally, recent findings suggest that sleep disordered breathing (SDB) and sleep movement disorders are more common in patients with PTSD than in the general population. These disorders may contribute to the brief awakenings, insomnia and daytime fatigue in patients with PTSD. This could be made worse by co-morbid alcoholism. SSRIs have a small but significant positive effect on sleep disruption. Nefazodone and trazodone lead to significant reductions in insomnia and nightmares, and Prazosin, a centrally acting α_1 -adrenoceptor antagonist, has led to large reductions in nightmares and insomnia in small studies. Cognitive behavioural interventions for sleep disruption in patients with PTSD include strategies targeting insomnia and imagery rehearsal therapy (IRT) for nightmares. RCT evidence exists for group IRT for significant reductions in nightmares and insomnia. Uncontrolled studies of continuous positive airway pressure for SDB in patients with PTSD show that this treatment led to significant decreases in nightmares, insomnia and PTSD symptoms.

Maher MM et al, "Sleep Disturbances in Patients with Post-Traumatic Stress Disorder: Epidemiology, Impact and Approaches to Management," CNS Drugs 20 (2006): 567-590.

Correct Answer is : 1. Decrease in alcohol intake 2. CBT 3. Decrease in alcohol intake 4. Change in sleep position, Continuous Positive Airway Pressure



Question 89

Not answered

Marked out of 4.00

Pharmacological management of personality

For each of the following types of symptoms, identify the evidence for pharmacological management in personality disorders

1. This class of drugs reduces aggressive, impulsive and angry behaviour in those with borderline and aggressive personality disorders.

✖

2. Identify 2 classes of drugs that have some effect against affective dysregulation in borderline disorder and aggressive outbursts in cluster B personality disorders

✖ ✖

3. Low dosages of these drugs might be effective in the treatment of both schizotypal and borderline personality disorders

✖

Explanation:

SSRIs reduce aggressive, impulsive and angry behaviour in those with borderline and aggressive personality disorders.

Anticonvulsants and lithium have some effect against affective dysregulation in borderline disorder and aggressive outbursts in cluster B.

Antipsychotics on low dosage might be effective in the treatment of both schizotypal and borderline personality disorders. They are useful for the treatment of symptoms such as suspiciousness, paranoid ideation, ideas of reference, magical thinking and stress-induced hallucinations.

Correct Answer is : 1. SSRIs 2. Lithium, Anticonvulsants 3. Antipsychotics

Question 90

Not answered

Marked out of 7.00

Management of eating disorders

Choose the best treatment in each of the following situations

1. A 23-year-old woman indulges in episodes of over eating followed by self-induced vomiting. She is also preoccupied with her weight all the time and regularly uses laxatives (Choose TWO)
2. A 16 year old girl with a history of eating disorder has been refusing food and restricting her fluids. She is expressing suicidal ideation. Her BMI is 15, pulse rate is 42/minute and her potassium is 2.4 (Choose TWO)
3. A 14-year-old girl has a low body weight with BMI of 17. She has a history of amenorrhoea and excessive fear of fatness.
4. A 27-year-old woman wishes to have ideal body shape and weight. She has a history of binge eating and depression. She is keen to take medications.
5. A 21-year-old girl has a history of comfort eating up to 1200 calories at one time. Her self-esteem is low due to her obesity. However she is not depressed.

Explanation:

Q1: The diagnosis is bulimia nervosa. Specifically adapted cognitive behavioural therapy should be offered to adults with bulimia nervosa, 16-20 sessions over 4-5 months. Interpersonal psychotherapy should be considered as alternative to cognitive behavioural therapy, but patients should be informed it takes 8-12 months to achieve similar results.

Q2: Hospital admission should be considered as there are serious medical problems in this girl with anorexia nervosa. Compulsory admission may be required and feeding is regarded as 'treatment'.

Q3: The diagnosis is anorexia nervosa (early onset). Family interventions that directly address the eating disorder are especially useful for children and adolescents with anorexia nervosa. SSRIs (specifically fluoxetine) are drugs of first choice for bulimia nervosa. Effective dose of fluoxetine is higher than for depression (60 mg daily). Antidepressant drugs can reduce frequency of binge eating and purging, but long-term effects are unknown.

Q4: The description fits a diagnosis of binge eating disorder. Binge eating disorder must be managed in a fashion similar to bulimia.

Q5: Binge-pattern of comfort eating can be addressed using CBT focused on eating behaviour.

Correct Answer is : 1. CBT, Interpersonal therapy 2. Hospital admission, Consider compulsory feeding 3. Family therapy 4. Fluoxetine 5. CBT



Question 91

Not answered

Marked out of 5.00

Breast feeding & psychotropic medication (1)

You have been asked to see a patient in a maternity ward. Mrs. Y is a 27-year-old woman who has delivered a baby 6 days ago and is breast-feeding. She is extremely anxious and hasn't slept for last 3 nights. She is agitated and expressing paranoid thought about nurses & midwives. A foundation year trainee working on this ward would like to seek your advice for further management. Choose the most appropriate drug for each question;

1. The preferred antipsychotics recommended for use (choose two)
2. The antipsychotic contraindicated during breast feeding
3. The sedative, which is safe for this woman as she had not slept for last 3 nights.
4. The ideal drug which could be used as an anxiolytic

Explanation:

The antipsychotics recommended for use in breast feeding mothers are olanzapine and sulpiride (Maudlsey guidelines-10th edition). There are no published data on Aripiprazole, Amisulpride, Sertindole and Ziprasidone.

Clozapine is contraindicated during breast-feeding as it accumulates in breast milk and foetal serum. Higher concentration of clozapine is due to higher concentration of albumin in foetal blood. Reported adverse effects include agranulocytosis (may recover spontaneously after discontinuation), decreased sucking reflex, drowsiness seizure, irritability and cardiovascular instability.

Zolpidem is safe during breast-feeding. Buspirone, zaleplon and zopiclone are excreted in breast milk and should be avoided.

Lorazepam is recommended for the treatment of anxiety in breast-feeding mothers. Diazepam and alprazolam should be avoided. Long acting benzodiazepine can produce lethargy, poor suckling and weight loss.

Correct Answer is : 1. Olanzapine, Sulpiride 2. Clozapine 3. Zolpidem 4. Lorazepam

Question 92

Not answered

Marked out of 6.00

Hepatic impairment

Identify the most appropriate and/or effective drug/drugs to be prescribed for the following clinical situations

1. Antipsychotics recommended to treat psychosis in a patient with hepatic impairment ✖
2. Antidepressants recommended to treat depression in a patient with hepatic impairment ✖
3. Mood stabilizer of choice in hepatic impairment ✖
4. Mood stabilizers contraindicated in severe liver disease (Choose TWO) ✖ ✖
5. Sedative of choice in severe liver disease ✖

Explanation:

Haloperidol (low dose) is considered as the drug of choice with sulpiride/amisulpiride considered as second choice options.

Among antidepressants, there is best evidence for use of Imipramine. Start with 25 mg/day and titrate slowly if required. Among SSRIs, Paroxetine and citalopram are recommended. Avoid sertraline.

Lithium is the mood stabilizer of choice in hepatic impairment, with gabapentin as second choice though the efficacy of the latter is questionable.

Mood stabilizers contraindicated in severe liver disease include sodium valproate and lamotrigine.

Where necessary, use only small dose of short acting benzodiazepines like lorazepam, oxazepam and temazepam. Sedative drugs can precipitate hepatic encephalopathy.

Correct Answer is : 1. Haloperidol 2. Imipramine 3. Lithium 4. Sodium valproate, Gabapentin 5. Oxazepam

Question 93

Not answered

Marked out of 7.00

Renal impairment

You have been asked to see 2 patients on renal dialysis unit with severe renal impairment and clinical presentations of psychotic and mood disorder respectively. The renal physician is keen to find out answers for the following questions

1. The antipsychotics which are safe to use in severe renal impairment (choose 2 answers)
2. The antipsychotics that should be avoided in patients with severe renal impairment (choose 2 answers)
3. The mood stabilizer that is relatively contraindicated in renal failure.
4. The antidepressants suggested as reasonable choices (choose 2 answers)

Explanation:

No antipsychotic agent is clearly preferred to another. The recommended antipsychotic drugs in renal impairment include olanzapine 5 mg a day, haloperidol 2-6 mg a day.

Clozapine is contraindicated in severe renal impairment. Avoid amisulpride and sulpiride in patients with renal failure.

Lithium is relatively contraindicated in renal failure. However its use may often be necessary and dose reduction (e.g. 50 to 75% for mild to moderate and 25-50% for severe renal failure) with close monitoring of plasma levels is recommended.

With respect to antidepressants, there is no clear winner that can be recommended and preferred to the others, however citalopram and sertraline are suggested as reasonable choices. In severe renal failure drugs like fluoxetine and venlafaxine should be avoided. (Ref: The Maudsley Prescribing Guidelines edn10- Pg 377)

Correct Answer is : 1. Haloperidol, Olanzapine 2. Clozapine, Amisulpride 3. Lithium 4. Citalopram, Sertraline



Question 94

Not answered

Marked out of 8.00

Epilepsy & psychotropics

You are seeing patients in the learning disability clinic. You have 2 patients, one with psychosis and the other with depression. Both have co-morbid epilepsy. A nurse is keen to know the medications that could be used in the following situations.

1. The antipsychotics with low pro-convulsive effects and are a good choice in treatments of psychotic symptoms (choose 2 answers)

2. The antipsychotics which are highly epileptogenic and should be avoided (choose 2 answers)

3. The antidepressants which are most epileptogenic and should be avoided (choose 2 answers)

4. The antidepressants which are safe in epileptic patients with clinical depression (choose 2 answers)

Explanation:

Haloperidol, Trifluoperazine and Sulpiride are the antipsychotics with low pro-convulsive effects and are a good choice in treatments of psychotic symptoms. Sulpiride has no known interactions with anti-convulsants.

Chlorpromazine and Clozapine are very epileptogenic and should be avoided. With clozapine, approximately 5% who receive more than 600 mg/day develop seizures. The risk is also high with other antipsychotics such as Loxapine, Zotepine and Olanzapine.

Most tricyclic antidepressants such as amitriptyline and dosulepin (dothiepin) appear to lower seizure threshold and are epileptogenic.

The antidepressants which are safe in epileptic patients with clinical depression are SSRIs and moclobemide. SSRIs are usually the first line drugs.

Correct Answer is : 1. Sulpiride, Haloperidol 2. Clozapine, Chlorpromazine 3. Amitriptyline, Dothiepin 4. SSRIs, Moclobemide



Question 95

Not answered

Marked out of 10.00

HIV Psychiatry

Choose 2 characteristic features for each of the following opportunistic infections in HIV patients

1. Cerebral Toxoplasmosis



2. Cryptococcal meningitis



3. Cytomegalovirus infection



4. Cerebral Tuberculosis



5. Progressive multifocal encephalopathy

**Explanation:**

Cerebral Toxoplasmosis is the most common pathogen in CNS, most common opportunistic infection in CNS and is due to the reactivation of a latent cerebral infection by *Toxoplasma gondii*, an opportunistic intracellular protozoan. The diagnosis is based on structural neuroimaging tests (multiple ring like calcified lesions) and treatment is with pyrimethamine and sulphadiazine.

Cryptococcal meningitis is due to yeast-like fungus *Cryptococcus neoformans*. It is characterized by headache, meningism, photophobia, nausea, fever, and delirium. CSF Indian ink staining, cryptococcal antigen titres and fungal culture can be decisive for the identification of cryptococcal meningitis.

Cytomegalovirus infection is the common cause of retinitis and there is 'Mozarella pizza' appearance on fundoscopy. It presents with peripheral neuropathy and demyelination.

Cerebral tuberculosis is the most common extra pulmonary infection in HIV and biopsy aids diagnosis.

Progressive multifocal encephalopathy is due to papovavirus infection affecting the white matter diffusely. Dementia can develop rapidly, with focal neurological alterations such as blindness, ataxia, and hemiparesis. Death follows very quickly thereafter, and there is no known treatment.

Correct Answer is : 1. Most common pathogen in central nervous system, CT/MRI shows multiple enhancing ring like lesions 2. Characterized by headache, meningism and photophobia, Indian ink staining can be decisive for identification 3. Common cause of retinitis, Mozarella pizza appearance on fundoscopy 4. Most common extra pulmonary infection, Biopsy aids diagnosis 5. Papovirus infection affecting white matter, Rapid onset dementia



Question 96

Not answered

Marked out of 9.00

Antipsychotics & side effects

A medical student has heard about antipsychotic medications and their side effects. He is keen to discuss options with you the suitability of certain drugs for patients with co-morbid medical conditions.

1. Which of the 2 drugs could be suggested for patients with dyslipidemia?
2. Which of the 2 drugs could be suggested for patients with impaired glucose tolerance?
3. Which drug is least likely to cause QT prolongation?
4. Which 2 drugs are least likely to cause hyperprolactinaemia?
5. Which 2 drugs are least likely to cause sexual side effects?

Explanation: The suggested drugs for patients with dyslipidemias are Amisulpride, aripiprazole and Ziprasidone. The suggested drugs for patients with impaired glucose tolerance are Amisulpride, aripiprazole and Ziprasidone. Aripiprazole is least likely to cause QT prolongation. The suggested drugs for patients with hyperprolactinaemia are Aripiprazole and Quetiapine. The suggested drugs for patients with sexual dysfunction are Aripiprazole and Quetiapine. (Ref: Maudlsey guidelines- 10th edition)

Correct Answer is : 1. Aripiprazole, Amisulpride 2. Aripiprazole, Amisulpride 3. Aripiprazole 4. Quetiapine, Aripiprazole 5. Quetiapine, Aripiprazole

Question 97

Not answered

Marked out of 8.00

Side effects of antipsychotics

A psychiatric nurse is concerned about side effects of antipsychotic medications. She would like to discuss medications, which can cause least side effects?

1. Which antipsychotic is least likely to produce dry mouth? ✖
2. Which 2 antipsychotics is most likely to cause weight gain? ✖ ✖
3. Which 2 antipsychotics have greatest risk of orthostatic hypotension? ✖ ✖
4. Which 2 antipsychotics have lowest risk of sedation? ✖ ✖
5. Which drug is least likely to cause tardive dyskinesia? ✖

Explanation:

Amisulpride does not cause anticholinergic side effects like dry mouth. The suggested drugs for patients with weight gain include amisulpride and aripiprazole. Other drugs of use include haloperidol and trifluoperazine. The suggested drugs for patients with postural hypotension are amisulpride, and aripiprazole. Other drugs, which are useful, would include haloperidol, trifluoperazine and sulpiride. The drugs, which have least sedative effects, include amisulpride and Aripiprazole. Other drugs would include risperidone and sulpiride. The drug which is least likely to induce tardive dyskinesia is clozapine. Other drugs which are less likely to induce tardive dyskinesia would include aripiprazole, olanzapine and quetiapine. (Ref: Maudsley guidelines- 10th edition)

Correct Answer is : 1. Amisulpride 2. Olanzapine, Clozapine 3. Risperidone, Clozapine 4. Amisulpride, Aripiprazole 5. Clozapine

Question 98

Not answered

Marked out of 9.00

Illness distributions

For each of the following descriptions, choose the disorders from the attached list that reflect key epidemiological observations.

1. Very high rates in males compared to females (Choose THREE options)

2. Equal prevalence in males and females (Choose THREE options)

3. Higher rates in females compared to males (Choose THREE options)

Explanation: Aspergers - 5:1 (DSM IV) Tourettes - 5:1 in clinical samples ; 2:1 in community samples (DSM IV) ASPD - 3:1 in community samples
Conversion disorder - 2:1 to 10:1 (DSM IV) Dissociative identity disorder - 3 - 9:1 (DSM IV) Panic disorder without agoraphobia - 2:1
Panic disorder with agoraphobia - 3:1 (DSM IV) Generalised anxiety : In epidemiological samples, 2/3 are females (DSM IV) Note that the actual ratios vary according to the study quoted.

Correct Answer is : 1. Asperger's syndrome, Tourette's disorder, Antisocial Personality Disorder 2. Anankastic personality Disorder, Bipolar disorder Type I , Schizoid Personality Disorder 3. Agoraphobia, Generalised Anxiety Disorder, Dissociative disorders

Question 99

Not answered

Marked out of 6.00

Risk Factors for self harm

Identify three most strongly associated risk factors for the following types of self-harm

1. Completed suicide      
2. Non-fatal self harm      

Explanation:

Self-harm occurs in people of all ages but unsurprisingly rates of self-harm are higher among those who are unemployed, single, live alone, are in debt and have problems with alcohol (National Collaborating Centre for Mental Health, 2004). Social isolation and breakdown in family and other personal relationships are regularly cited as causing self-harming behaviour (Haw & Hawton, 2008). People who are unemployed are two to three times more likely to die by suicide than people in employment (Platt, 2003; Rehkopf & Buka, 2006; O'Reilly et al, 2008), with unemployed men particularly at risk (Hawton et al, 2001). In addition, unemployment can itself contribute to poorer mental health, induce anxiety and depression, lower self-esteem and increase feelings of hopelessness - all of which increase the likelihood that someone will think that life is not worth living. People in debt and those who are homeless are particularly vulnerable (Shelter, 2004). Although adolescent girls are more likely than boys to harm themselves and overall women are more prone to harm themselves than men (Hawton & Harriss, 2008). The rate in young men aged 15-24 years is rising more quickly than in any other group. Older age itself does not increase the risk of suicide, but older people who harm themselves are more likely to do so in an attempt to end their life (National Collaborating Centre for Mental Health, 2004). Following an older person's self-harm the risk of completed suicide is also higher in their first year (60-100 times), which increases with age (Fox & Hawton, 2004; Hawton et al, 2007a). The only age group in England and Wales who are not showing a reduction in suicide rates are those over 85 years of age.

Correct Answer is : 1. Male gender, Unemployment, Hopelessness 2. Living alone, Unemployment, Alcohol use



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